

NIC CPD Micro-Credential

Professional Ethics, Safeguarding & Boundaries in Care

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 4–5 CPD learning hours

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Author: Manus AI

Important scope note: This course is professional development for caregivers and direct-care workers. It is not an introductory caregiving course, a legal qualification, a clinical licence, a specialist safeguarding qualification or a substitute for an employer's current safeguarding, privacy, disciplinary or incident-reporting procedures. Where a local law, public-authority direction, employer policy or care plan is more specific or more current, the learner must follow the applicable authorised instruction.

1. Course title

NIC CPD Micro-Credential: Professional Ethics, Safeguarding & Boundaries in Care

2. Course overview

This stand-alone CPD micro-credential strengthens the judgement and conduct required when a caregiver's work involves trust, dependency, personal information, intimate care, family relationships, money, digital communication or possible harm. It assumes that the learner already understands the basic role of a caregiver. The emphasis is therefore on applying professional principles in uncertain, pressured and emotionally complex situations rather than memorising introductory definitions.

The course is built around one central competency:

Recognise and respond appropriately to ethical, safeguarding and professional-boundary issues while protecting the dignity, rights, privacy and safety of clients and maintaining appropriate professional relationships.

Learners practise a repeatable decision sequence: **RECOGNISE → PAUSE → CONSIDER RIGHTS, SAFETY & PROFESSIONAL DUTY → DO NOT IGNORE → RESPOND WITHIN**

SCOPE → REPORT / ESCALATE → DOCUMENT OBJECTIVELY → MAINTAIN CONFIDENTIALITY.

The curriculum is relevant to home care, residential care, community work and healthcare-support environments in Nigeria. It respects the importance of family involvement, cultural and religious life, community relationships and multigenerational households while making clear that these contexts do not remove the client's dignity, rights, consent, privacy or safety. International references are used as best-practice resources and are not presented as Nigerian legal requirements or as evidence that the NIC credential is equivalent to another qualification [1](#) [2](#) [3](#).

Course at a glance

Element	Specification
Target learners	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and direct-support professionals
Course type	Stand-alone CPD micro-credential
Learning time	4–5 CPD hours
Structure	6 modules, 18 lessons, module assessments and a 35-question final assessment
Main method	Scenario-based online learning with reflective and judgement questions
Recommended final pass mark	80% (28/35)
Application emphasis	At least 23 of 35 final questions are scenario/application based
Practical limitation	Online completion demonstrates learning and scenario reasoning; it does not prove that a learner will always behave ethically in practice
Credential limitation	Does not create a regulated licence or specialist safeguarding status

3. Strategic purpose of the CPD

Caregivers are trusted with access to people's bodies, homes, routines, money, records, family relationships and private information. That trust creates responsibilities that are not captured by a checklist of tasks. A caregiver may need to respect a refusal of care while also recognising a safety concern; support a family without allowing the family to override the client's lawful and expressed wishes; respond to a disclosure without investigating; or decline money without humiliating the person who offered it.

This CPD is designed to reduce the gap between knowing a principle and acting on it under pressure. It supports NIC's wider caregiver education by developing professional judgement in boundary crossings, safeguarding concerns, conflicts of interest, digital communication, consent, confidentiality, dignity and reporting. It also supports a learning culture in which mistakes and near misses are reported early rather than concealed.

The course does not assume that a Nigerian family structure, faith tradition, language, age hierarchy or community relationship is inherently problematic. It teaches caregivers to work respectfully within those contexts while preserving the client's rights and the professional duty to protect against harm. Legal and regulatory details should be mapped by NIC and each employer to the relevant state, facility and service context before launch.

4. Target audience

The course is intended for working caregivers and related direct-care personnel who already have basic caregiving knowledge. It is especially suitable for people providing personal care, household support, community visits, companionship, mobility or daily-living assistance, delegated support under a care plan, family liaison, residential support or care supervision.

It is not intended to train learners to make formal legal determinations of mental capacity, conduct criminal or safeguarding investigations, provide psychotherapy, prescribe treatment, make clinical diagnoses, adjudicate complaints or replace a designated safeguarding lead, nurse, doctor, social worker, employer investigator or public authority.

5. Recommended CPD hours

NIC should publish **4–5 CPD learning hours**. A practical online allocation is approximately 3 hours for lessons, 45–60 minutes for lesson and module activities, and 30–60 minutes for the final assessment, review and completion record. The portal should record meaningful lesson completion, assessment attempts and acknowledgements rather than awarding credit merely for leaving a page open.

6. Prerequisites

The recommended prerequisite is **prior caregiving knowledge or experience**. NIC may accept completion of its Fundamental Caregiver programme, another recognised caregiver programme or evidence of current direct-care work. A mandatory prerequisite is not recommended because access should remain practical, but learners who lack basic understanding of the caregiver role should be directed to foundational training first.

7. Learning outcomes

By the end of the CPD, the learner should be able to:

1. **Apply** dignity, respect, autonomy, consent, privacy, confidentiality, non-discrimination, duty of care and accountability to everyday caregiving decisions.
2. **Distinguish** personal beliefs, cultural expectations and family preferences from professional responsibilities and client rights.
3. **Recognise** possible abuse, neglect, exploitation, inappropriate control, abandonment, self-neglect and boundary drift without making unsupported accusations or conducting an investigation.
4. **Respond** to disclosures or concerns by protecting immediate safety, listening appropriately, reporting through the authorised pathway and documenting objectively.
5. **Identify** when consent, refusal, privacy or decision-making concerns require additional support or escalation, without making formal legal capacity determinations outside role.
6. **Protect** confidential information in paper, electronic, telephone, WhatsApp, photographic and social-media contexts.
7. **Manage** gifts, money, bank cards, personal relationships, private communication and conflicts of interest in ways that protect clients from exploitation and caregivers from boundary risk.
8. **Demonstrate** professional conduct by following care plans, communicating reliably, admitting mistakes, refusing falsification and contributing to a learning culture.
9. **Use** the NIC ethical decision-making framework to recognise, pause, consider, respond, report, document and maintain confidentiality.

8. Competency framework

Domain	Expected capability	Evidence in this course
Rights and dignity	Preserves privacy, choice, respect and non-discrimination during routine and intimate care	Lessons 1–3 and scenarios
Ethical judgement	Separates personal preference from professional duty and weighs rights, safety and role	Lessons 1–2, final items 1–5
Consent and autonomy	Supports informed voluntary choice, respects refusal and escalates concerns appropriately	Lessons 4–5, final items 6–10
Confidentiality and digital professionalism	Uses minimum-necessary disclosure and secure information handling	Lessons 6–7, final items 11–16
Safeguarding recognition	Identifies indicators and vulnerability without diagnosing abuse or investigating	Lessons 8–10, final items 17–24
Professional boundaries	Detects boundary drift, manages gifts/money and avoids conflicts of interest	Lessons 11–13, final items 25–29
Accountability and response	Reports, documents, hands over and learns from incidents and errors	Lessons 14–18, final items 30–35

9. Professional decision-making framework

The NIC SAFE-CARE sequence

S — Stop and recognise. Notice the discomfort, inconsistency, request, disclosure, behaviour or risk. Do not dismiss a concern because it is inconvenient, culturally unfamiliar or raised by someone you do not expect.

A — Assess immediate safety and rights. Ask what could happen now, who may be at risk, what the client wants, whether urgent help is needed and whether the situation involves dignity, consent, privacy, exploitation or abuse.

F — Fit the response to your role and policy. Check the care plan, organisational procedure and authorised reporting route. Distinguish what you can do from what requires a supervisor, nurse, doctor, safeguarding lead, social worker, manager, police or other authorised service.

E — Engage calmly, without investigating. Listen, use open and respectful language, avoid leading questions, do not promise secrecy and do not confront a suspected perpetrator in a way that could increase risk.

C — Communicate, report and escalate. Tell the appropriate person promptly. Urgent danger requires immediate escalation, not a delayed form. Use the emergency and safeguarding pathways established by the employer or relevant authority.

A — Account for facts. Document what was observed, said, requested, done and reported, with time and date. Separate fact from interpretation and avoid unsupported labels.

R — Restrict disclosure. Share information only with people who need it to protect the client or manage the care, using approved channels. Keep records, photographs and digital messages secure.

E — Evaluate and follow up. Confirm that the concern reached the right person, follow the care plan and participate in review. Do not promise an outcome you cannot control.

Remember: A small boundary crossing can become a larger professional risk. Early pause and consultation are safer than waiting for harm to become obvious.

10. Complete module structure

Module	Title	Capability developed	Lessons	Assessment
1	Professional Values, Dignity and Ethical Judgement	Applies professional principles under pressure and distinguishes values from duty	1. Trust, Ethics and Professional Responsibility; 2. Personal Beliefs, Culture and Professional Duty; 3. Dignity, Respect and Person-Centred Practice	Five scenario and judgement questions

2	Client Rights, Consent and Autonomy	Supports choice, consent, refusal and independence while recognising when to seek help	4. Consent and the Right to Refuse; 5. Autonomy, Decision Support and Family Involvement; 6. Privacy, Intimate Care and Least-Restrictive Support	Six application questions
3	Confidentiality, Privacy and Digital Professionalism	Protects information across paper, electronic, WhatsApp, photography and social media	7. Confidentiality and Minimum-Necessary Disclosure; 8. Records, Devices and Digital Security; 9. Social Media, Images and Online Boundaries	Six application questions
4	Safeguarding and Protection from Harm	Recognises abuse, neglect, exploitation and vulnerability and responds within role	10. Safeguarding Recognition and Risk; 11. Abuse, Neglect, Exploitation and Indicators; 12. Responding to a Concern or Disclosure	Seven scenario questions
5	Professional Boundaries, Gifts and Conflicts	Prevents boundary drift, financial exploitation, favouritism and dual relationships	13. Boundary Drift and Relationship Risks; 14. Gifts, Money and Financial Safeguards; 15. Conflicts of Interest and Preferential Treatment	Six scenario questions
6	Professional Conduct,	Converts ethical judgement into reliable action,	16. Conduct, Reliability and Honest Practice;	Seven integrated questions

	Reporting and Accountability	reporting, documentation and learning	17. Reporting, Escalation and Documentation; 18. Integrated Ethical Decision-Making	
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11. Detailed lesson outline

Lesson	Purpose	Core application
1	Establish trust and ethics as daily professional practice	Decide what duty requires when personal convenience conflicts with client rights
2	Navigate personal, cultural and religious values	Respect context without imposing beliefs or allowing discrimination
3	Turn dignity into observable behaviour	Protect privacy, choice and respect during intimate and routine care
4	Apply consent and refusal principles	Explain, ask, respect, document and escalate
5	Support autonomy with family involvement	Keep the client central and seek appropriate guidance when concerns arise
6	Apply privacy and least-restrictive support	Protect bodily privacy and avoid coercive control
7	Use confidentiality and minimum-necessary disclosure	Respond to family and professional requests safely
8	Secure records and devices	Prevent unauthorised access, loss and casual sharing
9	Manage images, WhatsApp and social media	Think before capturing, sharing or posting
10	Recognise safeguarding concerns	Identify vulnerability and risk without investigation

11	Distinguish types and indicators of harm	Recognise patterns, not stereotypes or proof
12	Respond to disclosure or suspected harm	Protect, listen, report, document and preserve confidentiality
13	Detect boundary drift	Intervene early when relationships become personal or dependent
14	Manage gifts, money and financial risk	Refuse or disclose appropriately and protect client resources
15	Identify conflicts and favouritism	Keep decisions client-centred and transparent
16	Practise honest professional conduct	Report lateness, missed care, mistakes and falsification
17	Convert concern into escalation and records	Use authorised pathways and objective documentation
18	Integrate the full decision framework	Make safe, proportionate decisions in complex scenarios

12. Full lesson content

Module 1 — Professional Values, Dignity and Ethical Judgement

Lesson 1. Trust, Ethics and Professional Responsibility

Why this matters in professional caregiving

A client may allow a caregiver into spaces that are private even from close relatives. The caregiver may assist with bathing, toileting, dressing, meals, finances, medication support, transport or communication. The relationship is therefore built on trust, and trust creates a professional responsibility that continues even when no manager is watching. Ethical practice is not limited to dramatic dilemmas. It is visible in whether the caregiver knocks

before entering, tells the truth about a missed task, respects a client's preference and reports a concern rather than hoping someone else will notice.

Learning objectives

By the end of this lesson, the learner should be able to explain why trust creates professional duties; apply dignity, safety and accountability to a routine decision; identify a conflict between convenience and professional responsibility; and use the first steps of the NIC SAFE-CARE framework.

Core instructional content

Ethics is the disciplined process of deciding what ought to be done when more than one interest, value or duty is involved. In caregiving, a useful ethical question is: **What action best protects the client's dignity, rights, safety and wellbeing while remaining honest, within role and consistent with policy?** This question is more practical than simply asking what the caregiver personally prefers.

Professional responsibility includes reliability, respect, honesty, confidentiality, appropriate boundaries, accurate records, safe practice and willingness to escalate. These responsibilities apply in the client's home, in a residential facility and during community contact. The absence of a supervisor does not remove the duty. Nor does a family request automatically override the client's interests or the care plan.

A client-centred approach does not mean that every request can be granted. A client may ask the caregiver to do something outside role, conceal a risk, lend money, change a care-plan task or provide a private service. The caregiver can remain respectful while declining: "I understand why you are asking, but I cannot do that in my professional role. I will check the appropriate process with my supervisor." This protects the relationship without creating a hidden obligation.

Ethical practice also recognises power. The caregiver may control access to food, mobility support, private information, transport or contact with family. Even a polite request can feel compulsory to a dependent client. A caregiver should therefore avoid using access to care to obtain compliance, gifts, personal information, affection or silence. Threats, humiliation, deliberate delay and conditional kindness are professional risks even when no physical injury occurs.

Professional accountability is not perfection. Care work is complex and errors can occur. Accountability means acknowledging what happened, protecting the client, reporting promptly, correcting the record through the approved process and learning from the event. Concealment usually increases risk because the service loses the opportunity to respond. The principle for this course is: **A professional caregiver does not hide a mistake simply because admitting it is uncomfortable.**

Applying This in Practice

In **home care**, professional responsibility continues even when the family treats the caregiver as a guest or friend. In **residential care**, follow the care plan and handover even when a colleague says “we always do it this way.” In **hospital-support environments**, remain within delegation and escalate changes rather than improvising. In **community settings**, protect the same standard across clients; familiarity with one family does not justify special treatment.

Case scenario

A caregiver arrives late and discovers that a client’s breakfast and morning hygiene have not been completed. The caregiver is worried about being criticised and considers writing “care completed as planned” before rushing through the visit.

Question: What is the ethical issue, the immediate risk and the best response?

Correct professional response: The issue is honesty and accountability. The immediate risk is that the client’s needs remain unmet and the record becomes false. The caregiver should assess immediate safety, complete only the care that can be provided safely within the remaining time, notify the supervisor or service, record what was and was not done, explain the delay appropriately and arrange follow-up. The caregiver should not falsify the record or conceal the missed care.

Common professional risks

Common risks include treating a client as a task rather than a person, using dependency to obtain compliance, concealing mistakes, allowing family pressure to replace the care plan, making promises that cannot be kept, and assuming that an informal setting has no professional standards.

Key practice points

1. Trust is a professional responsibility, not a personal entitlement.
2. Convenience does not override dignity, safety or honesty.
3. Power must never be used to obtain gifts, silence or personal benefit.
4. Report and correct mistakes; do not falsify records.
5. When uncertain, pause and seek authorised guidance.

Lesson assessment

1. **MCQ:** Which best describes professional ethics? A) doing whatever feels kind; B) applying duties and rights to decide what should be done; C) obeying every family

request; D) avoiding difficult conversations. **Answer: B.** Ethics requires reasoned professional judgement. **Objective tested: 1.**

2. **Scenario:** A client asks the caregiver to hide a missed task. What should the caregiver do? **Answer:** Decline, report and document honestly. **Objective tested: 2–3.**
 3. **True/False:** A private home has no professional accountability. **Answer: False.** **Objective tested: 1.**
 4. **Best next action:** A caregiver is unsure whether a request is within role. **Answer:** Pause, check the care plan/policy and seek guidance. **Objective tested: 4.**
 5. **What should the caregiver NOT do?** Use delayed care or silence to pressure a client into compliance. **Answer:** Such conduct misuses power and may be abusive. **Objective tested: 2.**
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Lesson 2. Personal Beliefs, Culture and Professional Duty

Why this matters in professional caregiving

Caregivers bring their own faith, culture, family experience, opinions about ageing, disability, illness and appropriate behaviour to work. Clients and families also bring their own traditions and expectations. These differences can enrich care, but they may create tension when a personal belief conflicts with the client's preference or a professional standard. The solution is not to disparage culture or religion. It is to distinguish respectful cultural practice from imposing a personal belief, discriminating, ignoring consent or creating risk.

Learning objectives

The learner should be able to distinguish personal belief from professional responsibility; use respectful communication when values differ; identify discriminatory or coercive behaviour; and seek support when a belief conflict affects safe care.

Core instructional content

Personal values are beliefs about what is important or acceptable. Professional responsibilities are duties attached to the caregiver role, including respect, non-discrimination, confidentiality, consent, safety, honesty and following authorised care instructions. A caregiver may hold a religious belief about diet, gender roles or family authority, but cannot use that belief to shame a client, deny routine care within role, disclose private information or override a client's lawful choice.

Respecting culture means asking rather than assuming. A caregiver can ask, "Are there practices or preferences that would help you feel comfortable during this care?" The

client's answer should be considered alongside safety, the care plan and the caregiver's scope. If a request cannot safely or professionally be met, explain the limitation and seek an appropriate alternative. Cultural respect is not the same as agreeing to every practice or allowing a family member to control a competent client's choices.

Discrimination may be direct, such as refusing care because of disability, religion, gender identity, ethnicity, age, health condition or socioeconomic status. It may also be indirect, such as using a procedure that makes access unnecessarily difficult for a person with communication or mobility needs. Disrespectful jokes, infantilising language and assumptions that an older person cannot decide are also professional risks.

Family authority is important in many Nigerian households, but the caregiver should not assume that the loudest or oldest person automatically has authority to receive information or make decisions for the client. The caregiver should follow consent, the care plan and organisational policy, and seek qualified guidance when a family request conflicts with the client's expressed wishes or safety.

A values conflict should be managed early. The caregiver should recognise the tension, pause, identify the client's rights and safety, check role and policy, and seek supervision. If the caregiver cannot perform a task because of a genuine conscientious conflict, the caregiver must not abandon the client or obstruct access to care; the service should manage the issue through its authorised process.

Applying This in Practice

In **home care**, ask about household preferences while keeping the client's voice central. In **residential care**, do not impose a uniform routine when reasonable individual choice is possible. In **hospital-support settings**, follow professional and facility standards even when family customs differ. In **community settings**, use interpreters or authorised communication support where needed rather than relying on a child or neighbour for sensitive information.

Case scenario

A caregiver believes that a client with a disability should not make decisions about clothing and asks the client's sibling to choose everything. The client repeatedly indicates a preference for a different outfit.

Correct professional response: The caregiver should respect the client's communication and preference, provide reasonable support and follow the care plan. If there is a genuine concern about decision-making or safety, the caregiver should document observations and seek qualified guidance, not assume incapacity or allow a personal belief to decide.

Common professional risks

Risks include religious or cultural imposition, ageism, disability-based assumptions, using a relative as an interpreter for confidential matters without authorisation, treating family hierarchy as automatic decision-making authority, and using “culture” to excuse coercion or discrimination.

Key practice points

1. Personal belief is not a licence to override professional duty.
2. Ask about preferences; do not stereotype.
3. Family involvement can support care but does not automatically replace the client’s voice.
4. Respectful challenge is safer than silent compliance.
5. Escalate genuine conflicts rather than abandoning care.

Lesson assessment

1. **Scenario:** A family asks the caregiver to hide a diagnosis from a client who asks directly.
Answer: Do not promise concealment; explain that information-sharing must follow consent and authorised clinical communication, and seek supervisor/clinician guidance. **Objective tested: 2–3.**
 2. **MCQ:** Which is professional? A) assume a religious preference; B) ask respectfully and check safety/policy; C) mock an unfamiliar practice; D) let family pressure decide.
Answer: B. Objective tested: 2.
 3. **True/False:** Respecting culture requires accepting any request made in the name of culture. **Answer: False.** Rights and safety remain relevant. **Objective tested: 2–3.**
 4. **What should the caregiver NOT do?** Assume disability means inability to choose.
Answer: Observe and support; do not make an unsupported determination. **Objective tested: 3.**
 5. **Best next action:** A client’s preference conflicts with a family routine but presents no safety issue. **Answer:** Centre the client’s preference and seek agreement through respectful communication. **Objective tested: 2.**
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Lesson 3. Dignity, Respect and Person-Centred Practice

Why this matters in professional caregiving

Dignity is not simply being friendly. It is the practical protection of a person’s worth, privacy, identity, choices and right to be treated without humiliation. Care tasks such as bathing, toileting, dressing, feeding and continence support can either strengthen or

undermine dignity. Small actions—closing a door, using a preferred name, asking before touching, covering the body and explaining a delay—can determine whether the client feels safe.

Learning objectives

The learner should be able to identify dignity risks in routine and intimate care; use respectful communication and privacy measures; support personal choice and cultural preference; and recognise when convenience has become humiliating or controlling.

Core instructional content

Dignity begins with seeing the client as a person rather than a diagnosis, age, disability or task list. Use the person's preferred name and communication method. Speak to the client, not only to a relative or colleague. Explain what will happen before touching or moving the person. Ask permission where possible and pause when the client appears distressed.

Intimate care requires privacy and preparation. Close doors or curtains, limit unnecessary observers, gather supplies first, use appropriate covering and expose only the area needed for the task. Do not discuss toileting, continence, sexuality, bodily functions or personal history where other people can hear. A client's home is not a public space simply because relatives are present.

Dignity includes choice. Offer reasonable choices about clothing, bathing time, food presentation, music, personal items and who is present. Choice must be adapted to communication, sensory, cognitive or mobility needs. A caregiver should not rush a client into agreement because the schedule is tight. If a decision has safety implications, explain the concern, seek guidance and document rather than using ridicule or threat.

Feeding and mobility can carry dignity risks. Do not force food, feed in a humiliating way, talk about the client as if they are absent or leave a person exposed after a transfer. If the client refuses food or support, explore the reason within role, respect the refusal unless immediate danger requires escalation, and notify the appropriate professional according to the care plan.

Respect also applies to possessions, appearance and identity. Do not use client property without permission, rearrange personal items unnecessarily, comment negatively on the home or make jokes about clothing, body, accent, age or disability. A person-centred approach is compatible with professional boundaries: warmth and respect do not require friendship, secrecy or personal disclosure.

Applying This in Practice

In **home care**, protect privacy even when family members are nearby. In **residential care**, avoid exposing residents in corridors or leaving personal items unidentified. In **hospitals**, use curtains and appropriate communication during transfers. In **community settings**, consider privacy in vehicles, waiting areas and public conversations.

Case scenario

A caregiver is assisting a client with toileting. A relative enters the room and begins discussing the client's incontinence loudly. The client looks embarrassed but does not speak.

Correct professional response: Protect privacy, ask the relative respectfully to allow private care and speak to the client in a dignified manner. After the task, check how the client wishes information to be shared and report any recurring privacy problem. The caregiver should not join the conversation or treat silence as consent.

Common professional risks

Risks include exposing the body unnecessarily, speaking over the client, infantilising adults, discussing private matters publicly, forcing food or care, using client property, mocking disability or age, and allowing time pressure to become humiliation.

Key practice points

1. Dignity is shown through observable actions.
2. Ask before touching and explain before acting.
3. Expose only what is necessary during intimate care.
4. Offer choices and support communication.
5. Warmth does not require a private friendship.

Lesson assessment

1. **Scenario:** List three dignity protections during bathing. **Answer:** Explain and seek permission; close the door/curtain; expose only the area being washed; use preferred communication; maintain covering and respectful language. **Objective tested: 1–2.**
2. **MCQ:** Which is a dignity breach? A) offering clothing choices; B) discussing continence in front of visitors; C) asking before transfer; D) covering the client. **Answer: B. Objective tested: 1.**
3. **True/False:** A silent client has automatically consented to a relative hearing private information. **Answer: False. Objective tested: 2.**

4. **What should the caregiver NOT do?** Force food because the schedule is busy. **Answer:** Explore, respect, document and escalate according to the care plan. **Objective tested:** 3.
5. **Best next action:** The client appears distressed during intimate care. **Answer:** Pause, reassure, ask what is wrong and seek guidance if the task cannot safely continue. **Objective tested:** 1–3.
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Module 2 — Client Rights, Consent and Autonomy

Lesson 4. Consent and the Right to Refuse

Why this matters in professional caregiving

Consent is not a formality. It is the client's opportunity to understand what is proposed, make a voluntary choice and change their mind. Caregivers often provide personal or intimate support, so consent must be treated as an ongoing interaction. A client's refusal may be inconvenient, but it is information that must be respected and explored within role rather than overridden through force, threats, shame or deception.

Learning objectives

The learner should be able to explain consent in practical terms; seek consent before routine and intimate care; respond safely to refusal; distinguish refusal from an urgent emergency; and document and escalate concerns without making a formal capacity determination.

Core instructional content

For caregiving purposes, consent should be **informed enough for the task, voluntary, specific to the activity and open to withdrawal**. The caregiver should explain what they intend to do in language the client can understand, check whether the client agrees and observe verbal and non-verbal communication. Consent to one task does not automatically authorise another. Consent to bathing does not mean consent to photographs, sharing information or a family member staying in the room.

Communication should be adapted. Use plain language, visual prompts, an interpreter or other authorised support where needed. Do not ask a leading question such as “You want me to do this, don't you?” Give reasonable time. If the client seems confused or distressed, pause and seek guidance. Caregivers should observe and report; they should not make formal legal determinations about mental capacity unless explicitly authorised and trained under applicable policy.

A refusal can have many causes: pain, fear, embarrassment, fatigue, cultural preference, previous trauma, misunderstanding, loss of control, depression, delirium, medication effects or a desire for privacy. Ask an open question such as, “Can you tell me what is worrying you?” Do not interrogate. Offer choices where possible and return later if the care plan allows. If the refusal creates immediate risk, follow the emergency or clinical escalation pathway.

Force, threats, withholding food or care, deliberate delay, mocking and deceptive consent are unacceptable. A caregiver should not say “If you do not cooperate, I will tell your family” or “This will only take a second” when the task has not been explained. If a task is essential but refused, the caregiver should notify the responsible professional and document the refusal, the information provided, the client’s words or behaviour and the response.

Family members may assist communication, but they should not pressure the client. If a family member says “Do it anyway,” the caregiver should state that the client’s wishes and the authorised care plan must be respected and seek supervisor or clinical guidance.

Applying This in Practice

In **home care**, allow privacy and time rather than treating family routine as consent. In **residential care**, document refusals and communicate them at handover. In **hospitals**, follow clinical escalation for refusal of delegated support or deterioration. In **community settings**, ensure interpreters or communication supports are authorised and confidential.

Case scenario

A client refuses a shower and says, “Not today; I am in pain.” The client’s daughter tells the caregiver to ignore the refusal because the client “always complains.”

Correct professional response: Acknowledge the refusal, ask about the pain within role, offer reasonable alternatives such as a later time or partial wash if the client agrees, and notify the responsible professional if pain or missed hygiene creates concern. Do not force the shower or allow the daughter to override the client without authorised basis.

Common professional risks

Risks include treating silence as consent, rushing explanations, using family pressure, confusing care-plan routine with permission, forcing or deceiving, ignoring non-verbal distress and recording “refused” without documenting the circumstances.

Key practice points

1. Consent is task-specific, voluntary and can be withdrawn.

2. Explain, ask, listen and allow time.
3. A refusal is not misconduct.
4. Do not make formal capacity decisions outside role.
5. Report refusals that create safety or care concerns.

Lesson assessment

1. **MCQ:** Consent should be: A) assumed for every task; B) informed enough, voluntary and open to withdrawal; C) provided by any relative; D) obtained once for life. **Answer: B.**
Objective tested: 1.
 2. **Scenario:** A client refuses personal care because of pain. **Answer:** Respect, explore within role, offer safe alternatives and escalate the pain/refusal concern. **Objective tested: 2–4.**
 3. **True/False:** A family member may always override a client's refusal. **Answer: False.**
Objective tested: 3.
 4. **What should the caregiver NOT do?** Threaten to withhold care or expose the client to family shame. **Answer:** Coercion undermines voluntary choice and may be abusive.
Objective tested: 3.
 5. **Documentation question:** What belongs in the record? **Answer:** Task, time, client's words/behaviour, explanation given, alternatives offered, action taken and person informed. **Objective tested: 5.**
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Lesson 5. Autonomy, Decision Support and Family Involvement

Why this matters in professional caregiving

Autonomy means supporting a person to direct their own life and care as far as possible. It does not require a client to make every decision without help, and it does not mean that the caregiver ignores serious risk. The professional task is to support understanding and choice, respect the client's preferences, recognise when additional guidance is needed and avoid allowing family convenience to replace the client's voice.

Learning objectives

The learner should be able to support independence; identify when a client may need communication or decision support; manage family pressure; distinguish observation from legal determination; and escalate concerns appropriately.

Core instructional content

Supporting autonomy can involve offering manageable choices, arranging the environment so the client can participate, allowing extra time, using communication aids and asking what matters to the person. Independence is not all-or-nothing. A client may choose clothing, food, visitor arrangements or the order of tasks even while receiving substantial physical support.

Decision support begins with listening. Do not assume that a person who communicates slowly, uses a wheelchair, has dementia, has a mental-health condition or is older cannot express a preference. Do not ask every question through a relative. Use the client's preferred language and communication method, and check understanding without humiliation.

A caregiver may notice concerns: the client gives different answers at different times, appears frightened when a relative is present, cannot understand an important instruction, or shows a sudden change from baseline. The caregiver should document the observations and seek the authorised professional assessment. The caregiver should not label the client incapable, sign on the client's behalf without authority, or use a family member's opinion as proof.

Family involvement can be helpful in understanding routines, culture, language and history. It can also create pressure, conflict or information risks. When a family member demands a choice that conflicts with the client's expressed preference, the caregiver should stay neutral, protect the client's privacy, state that the issue requires the authorised care pathway and escalate. The caregiver should not take sides or promise the family a particular decision.

Autonomy must be balanced with safety and duty of care. If a client chooses an option that is inconvenient but safe, respect it. If the choice creates immediate serious danger, take reasonable immediate protective action within training and seek urgent guidance. Avoid blanket restrictions simply because they are easier for staff.

Applying This in Practice

In **home care**, use the client's daily routines as a starting point and negotiate safe changes. In **residential care**, avoid imposing a single timetable when alternatives are feasible. In **hospital-support work**, communicate changes in preferences or behaviour to the nurse or supervisor. In **community settings**, use authorised interpreters and never rely on a child to handle sensitive consent discussions.

Case scenario

A client's son instructs the caregiver not to tell the client about an upcoming change in care because it will "confuse him." The client asks, "What is happening tomorrow?"

Correct professional response: Do not promise the son secrecy or provide information beyond role. Acknowledge the client's question, explain that the responsible professional should discuss care changes, notify that professional and document the request and the client's question. The caregiver should not lie or disclose confidential family conflict.

Common professional risks

Risks include speaking only to relatives, assuming cognitive impairment equals incapacity, substituting family convenience for client choice, restricting movement for staff convenience, signing for a client without authority and failing to report a change in decision-making ability or behaviour.

Key practice points

1. Support the client to participate; do not assume inability.
2. Family involvement is not automatic decision-making authority.
3. Observe, document and escalate; do not make legal determinations.
4. Use the least restrictive safe option.
5. Protect the client from pressure without humiliating the family.

Lesson assessment

1. **Scenario:** A client communicates slowly but clearly chooses their clothing. **Answer:** Allow time, support communication and honour the preference. **Objective tested: 1–2.**
 2. **MCQ:** A caregiver's role when concerned about decision-making is to: A) declare incapacity; B) observe, document and seek guidance; C) ask a neighbour; D) ignore it. **Answer: B. Objective tested: 3.**
 3. **True/False:** Family information can be useful but should not automatically replace the client's voice. **Answer: True. Objective tested: 2–3.**
 4. **What should the caregiver NOT do?** Restrict a client's choices solely because it is easier for staff. **Answer:** Use the least restrictive safe option and escalate genuine risk. **Objective tested: 4.**
 5. **Best next action:** A family member demands secrecy from the client. **Answer:** Do not promise secrecy; refer the issue to the appropriate professional and document objectively. **Objective tested: 3–5.**
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Lesson 6. Privacy, Intimate Care and Least-Restrictive Support

Why this matters in professional caregiving

Privacy is a bodily, personal and informational right. Intimate care can make a client especially vulnerable to embarrassment, exposure and coercion. A professional caregiver should use the least intrusive approach that safely achieves the task, preserve privacy and recognise that family presence, staff routines and busy environments can gradually normalise unnecessary exposure.

Learning objectives

The learner should be able to plan privacy during intimate care; identify restrictive or controlling practices; seek consent at each stage; and report recurring practices that undermine dignity or autonomy.

Core instructional content

Before intimate care, prepare the space, supplies and people needed. Explain the task, ask who the client wants present and close doors or curtains. Use appropriate draping and expose only the body area required. Do not leave the client uncovered while searching for supplies or discussing the person with another worker.

Privacy also includes the right to personal space and private communication. Do not read a client's messages, open drawers, enter a room without knocking or listen to a private conversation unless a clear care need and authorised process require it. Do not use a client's toilet, clothing, bed or personal items for convenience.

Restrictive practice includes physical restraint, unnecessary confinement, threats, removal of communication devices, controlling access to visitors or money, or using medication or sedation for behaviour control. Caregivers should not initiate or improvise restraint. If a client is at immediate risk of harming themselves or another person, follow the emergency plan and seek qualified assistance. A restrictive measure, if authorised at all, requires proper assessment, documentation, oversight and review by the responsible professionals.

Least-restrictive practice means asking what support would achieve safety with less intrusion. A client who wanders may need environmental adjustment, conversation, meaningful activity or clinical review rather than locked doors. A client who refuses bathing may need a different time or method rather than force. Caregivers should report recurring restrictions and speak up if "routine" is being used to justify humiliation or control.

Privacy and dignity must be balanced with safeguarding. If a caregiver needs to interrupt privacy to respond to immediate danger, do so proportionately, explain afterwards and report. The goal is not absolute privacy at any cost; it is respectful, necessary and safe privacy.

Applying This in Practice

In **home care**, agree how family members will respect private care. In **residential care**, use curtains, doors and appropriate staff numbers. In **hospitals**, follow chaperone and transfer procedures. In **community work**, protect private conversations in vehicles and public spaces.

Case scenario

A facility routinely leaves bathroom doors open so staff can “keep an eye” on residents, even when the resident can use the toilet independently.

Correct professional response: Recognise the dignity and privacy risk, raise the concern with the supervisor and suggest proportionate alternatives such as agreed checks, call systems or individual risk plans. The caregiver should not silently continue an unnecessary practice or close doors in a way that creates a real safety risk without discussion.

Common professional risks

Risks include unnecessary observers, open doors, inadequate draping, routine restraint, controlling visitors without authority, reading personal messages, using property without permission and treating surveillance as the default solution.

Key practice points

1. Prepare privacy before intimate care begins.
2. Use the least intrusive safe option.
3. Do not improvise restraint or confinement.
4. Privacy can be interrupted only proportionately for safety.
5. Report routine practices that undermine rights.

Lesson assessment

1. **Scenario:** The client is left uncovered while the caregiver searches for supplies.
Answer: Cover the client, prepare supplies before care and report recurring workflow problems. **Objective tested: 1.**
2. **MCQ:** Least-restrictive support means: A) no safety measures; B) the safest effective option that interferes least with rights; C) maximum surveillance; D) physical restraint first. **Answer: B. Objective tested: 3.**
3. **True/False:** Caregivers may improvise restraint if a client is difficult. **Answer: False. Objective tested: 3–4.**
4. **What should the caregiver NOT do?** Read a client’s private messages out of curiosity. **Answer:** It is an unauthorised privacy breach. **Objective tested: 2.**

5. **Best next action:** An open-door practice is not needed for a client who can toilet safely.
Answer: Raise the concern and seek a proportionate individualised arrangement.
Objective tested: 4–5.
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Module 3 — Confidentiality, Privacy and Digital Professionalism

Lesson 7. Confidentiality and Minimum-Necessary Disclosure

Why this matters in professional caregiving

Clients share information because they expect it to be handled carefully. A caregiver may know about illness, finances, family conflict, disability, medication, relationships, trauma or personal routines. Casual disclosure can cause humiliation, discrimination, family conflict or exploitation. Confidentiality does not mean refusing every request for information; it means sharing only what is authorised and necessary for safe care through an appropriate channel.

Learning objectives

The learner should be able to distinguish privacy from confidentiality; apply minimum-necessary disclosure; respond to family and professional requests; and escalate uncertainty rather than guessing.

Core instructional content

Privacy concerns a person's control over access to their body, space and information. **Confidentiality** concerns the caregiver's duty to protect information received through the professional relationship. Information can be confidential even when it is true, already known by some relatives or visible in the home.

Use the minimum-necessary principle: share the smallest amount of relevant information with the right person for a legitimate care purpose. A caregiver may need to tell a supervisor that a client refused care and appeared distressed. The caregiver may not need to describe unrelated family history. A receiving nurse may need an objective symptom report; a neighbour does not.

Family requests require care. A relative may be closely involved but not automatically authorised to receive every detail. The caregiver should check the care plan, consent arrangements and employer policy. If uncertain, say, "I want to protect your relative's privacy, so I need to check what I am authorised to share." Do not invent a rule, disclose out of politeness or use a personal phone for convenience.

Information may be shared without the client's ordinary permission in limited circumstances established by law, policy, safeguarding or urgent safety needs. Caregivers should not make that legal decision independently. They should follow the authorised pathway and document what was shared, with whom, why and under whose direction.

Confidentiality continues outside working hours. Do not discuss clients in transport, restaurants, religious gatherings, salons, family homes or online groups. Use initials or a first name only where the approved system requires it, but remember that a combination of details can identify a person. A de-identified story may still be recognisable in a small community.

The Nigeria Data Protection Commission identifies the Nigeria Data Protection Act, 2023 and related privacy resources as part of Nigeria's data-protection framework ⁴. This course does not interpret the Act for every care situation. NIC and employers should map current legal and organisational requirements, train staff on approved channels and provide a clear breach-reporting route.

Applying This in Practice

In **home care**, agree who may receive updates and how. In **residential care**, use authorised handover and records rather than corridor conversations. In **hospitals**, disclose through approved clinical channels. In **community care**, do not discuss one household with another, even when families know each other.

Case scenario

A client's sister asks the caregiver to send daily medical updates to her personal WhatsApp number. The client has not provided permission and the caregiver is unsure what the care plan says.

Correct professional response: Do not start sending updates. Check consent and the approved communication process with the supervisor or responsible professional. Share only authorised, necessary information through a secure channel and document the request and direction received.

Common professional risks

Risks include assuming family access, discussing cases in public, sharing more than necessary, using personal messaging accounts, sending information to the wrong contact, treating gossip as harmless and confusing "everyone already knows" with authorisation.

Key practice points

1. Confidentiality is a continuing professional duty.

2. Share the minimum necessary information with the right person.
3. Family closeness does not automatically equal authorisation.
4. Use approved channels and seek guidance when uncertain.
5. Protect information in small communities as carefully as in institutions.

Lesson assessment

1. **MCQ:** Minimum-necessary disclosure means: A) tell everyone the whole story; B) share only relevant information with an authorised person; C) share nothing ever; D) post anonymously. **Answer: B. Objective tested: 1–2.**
 2. **Scenario:** A family member asks for medical details without clear authorisation. **Answer:** Pause, check consent/policy and refer to the authorised professional. **Objective tested: 3.**
 3. **True/False:** A first name always makes information anonymous. **Answer: False.** Context can identify the client. **Objective tested: 2.**
 4. **What should the caregiver NOT do?** Use a personal WhatsApp account because it is quicker. **Answer:** Follow the approved communication route. **Objective tested: 4.**
 5. **Documentation question:** What should be recorded after an uncertain information request? **Answer:** Request, date/time, information sought, direction received and what was disclosed, if anything. **Objective tested: 5.**
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Lesson 8. Records, Devices and Digital Security

Why this matters in professional caregiving

A lost notebook, unlocked phone, shared password or screenshot can expose a client's identity and private information. Digital professionalism is not only an information-technology issue. It is part of safeguarding because information can be used to shame, exploit, locate or pressure a vulnerable client. Caregivers should use secure systems, protect access and report loss or misdirection promptly.

Learning objectives

The learner should be able to keep paper and electronic records secure; use passwords and devices responsibly; identify a possible data breach; and report digital incidents through the authorised route.

Core instructional content

Records should be accurate, timely, relevant and stored where authorised people can access them. Do not leave paper records on a car seat, kitchen table, reception desk or shared printer. Do not take records home unless the employer authorises it. Dispose of drafts through the approved confidential route rather than ordinary household waste.

Use individual passwords and never share them. Lock the screen when stepping away. Do not store client information in personal notes, contact lists, cloud drives or photo galleries unless the system is approved. Be cautious with autocomplete, group messages and contact names; sending information to the wrong person is a common preventable error.

A strong password, device lock and approved application reduce risk but do not make careless sharing acceptable. Personal devices may be lost, stolen, infected or viewed by relatives. If personal-device use is authorised, follow the organisation's controls and do not copy information to other apps. If it is not authorised, do not use it for client information even if a family member requests convenience.

A possible breach includes a lost phone containing client information, a message sent to the wrong contact, an unauthorised person viewing a record, a screenshot in a group chat, a stolen paper file or a device left unlocked. Report promptly through the employer's incident or privacy pathway. Do not delete messages, conceal the mistake or attempt an unauthorised investigation. Preserve relevant information as instructed.

Records should distinguish fact from opinion. "Client stated that money was missing" is different from "Family stole money." Record the source, date, time and action taken. A record can support safeguarding, continuity and accountability only if it is objective and secure.

Applying This in Practice

In **home care**, keep records away from household visitors and children. In **residential care**, use approved electronic records and log out of shared terminals. In **hospitals**, follow the facility's device and access-control rules. In **community work**, secure records during transport and report lost items immediately.

Case scenario

A caregiver accidentally sends a client's appointment screenshot to a family group containing people who are not involved in the client's care.

Correct professional response: Stop further forwarding, do not delete evidence unless instructed, notify the supervisor/privacy contact immediately, record what was sent and to whom, follow breach instructions and avoid making additional disclosures while trying to fix it informally.

Common professional risks

Risks include shared passwords, unlocked screens, screenshots, cloud backups, paper records in public places, storing client information in personal contacts, wrong-recipient messages and delayed breach reporting.

Key practice points

1. Secure information at rest and in transit.
2. Do not share passwords or use unauthorised storage.
3. Report errors promptly; do not conceal or delete evidence.
4. Write objective records.
5. Treat a digital breach as a professional incident.

Lesson assessment

1. **MCQ:** The safest action when leaving a device is to: A) leave it open; B) lock it; C) give the password to a friend; D) photograph the screen. **Answer: B. Objective tested: 2.**
 2. **Scenario:** A paper record is left in a public vehicle. **Answer:** Retrieve or secure it if safe, report immediately through the privacy/incident pathway and document the event. **Objective tested: 3.**
 3. **True/False:** Deleting an incorrect message always resolves a privacy breach. **Answer: False.** Reporting and authorised response are required. **Objective tested: 3.**
 4. **What should the caregiver NOT do?** Store client details in a personal cloud drive for convenience. **Answer:** Use only approved systems. **Objective tested: 2.**
 5. **Documentation question:** Which is objective? A) “The family is dishonest” ; B) “Client reported missing cash at 16:00; supervisor informed at 16:15.” **Answer: B. Objective tested: 5.**
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Lesson 9. Social Media, Images and Online Boundaries

Why this matters in professional caregiving

Photographs, video, geolocation, social-media posts and private messages can turn a private care relationship into a public exposure. A caregiver may think a picture is harmless because the client’s name is absent, but the home, face, date, clothing, location or story may identify the person. Modern professionalism requires a deliberate pause before capturing, sharing or posting.

Learning objectives

The learner should be able to apply the “think before you capture, share or post” rule; identify risks in photography and messaging; respond to friend requests and online complaints; and protect clients from digital exposure.

Core instructional content

Do not photograph or record a client, wound, medication, room, document, home, family member or care event using a personal device unless there is a clear authorised purpose, valid process and secure approved system. A family member’s request does not automatically create permission. Client consent must be meaningful, but consent alone may not make a personal channel or public post appropriate. Follow employer policy and authorised clinical or safeguarding processes.

A photograph can reveal more than intended: a face reflected in a mirror, a house number, a uniform, a location tag, a hospital bracelet or a distinctive family event. Cropping or removing a name may not make the person anonymous. Screenshots can be forwarded, copied or stored indefinitely. WhatsApp groups may include people whose role has changed or who are not authorised to receive information.

Friend requests and private messaging create boundary risk. A caregiver should not connect with clients through personal social-media accounts or use private messaging for unapproved care communication. If a client sends a request, respond courteously through the approved channel or explain the professional boundary. Do not accept a request to “keep in touch after work” without checking policy; do not move a client relationship into a private friendship.

Online complaints should be handled through the organisation’s complaint process. Do not argue publicly, reveal client details, post a defensive story or ask friends to support the caregiver online. If the caregiver sees confidential information posted by someone else, do not forward it; report it through the appropriate route and protect the client.

The course rule is:

THINK BEFORE YOU CAPTURE → THINK BEFORE YOU SHARE → THINK BEFORE YOU POST.

If a photograph or message was sent incorrectly, report immediately. Do not rely on deleting it, asking recipients to ignore it or claiming good intentions.

Applying This in Practice

In **home care**, do not post pictures of the home, family celebrations or client belongings. In **residential care**, follow facility rules for photography, visitors and staff devices. In **hospitals**, use only approved record systems. In **community settings**, disable unnecessary location sharing and avoid identifying the route between clients.

Case scenario

A caregiver photographs a client's wound "for record keeping" and plans to send it to the client's daughter from a personal phone.

Correct professional response: Do not use a personal phone or send the image through an unapproved channel. Follow the authorised clinical documentation process and ask the responsible professional what is required. If the image has already been taken, report it and preserve it according to instruction rather than forwarding or deleting it casually.

Common professional risks

Risks include posting from a client's home, accepting friend requests, sharing screenshots, location tags, recording without proper authorisation, using personal WhatsApp, discussing cases in groups and treating "no name included" as anonymity.

Key practice points

1. A good intention does not make an unauthorised image safe.
2. Personal devices and social-media accounts are not default care systems.
3. De-identification may fail in a small community.
4. Do not respond to complaints with public disclosure.
5. Report digital mistakes promptly.

Lesson assessment

1. **Scenario:** A client sends a friend request. **Answer:** Do not accept through the personal account; maintain the boundary and use approved communication. **Objective tested:** 2.
2. **MCQ:** Removing a client's name from a photo: A) always anonymises it; B) may not prevent identification; C) makes posting acceptable; D) replaces consent. **Answer: B.** **Objective tested:** 2.
3. **True/False:** A family request automatically authorises a caregiver to photograph a client. **Answer: False.** **Objective tested:** 1–2.
4. **What should the caregiver NOT do?** Argue about a complaint on social media using care details. **Answer:** Use the authorised complaints process. **Objective tested:** 4.
5. **Best next action:** A wound photo is already on a personal device. **Answer:** Report immediately and follow authorised instructions; do not forward or casually delete it. **Objective tested:** 5.

6. **Scenario:** A WhatsApp group includes former staff and unrelated relatives. **Answer:** Do not share client information there; notify the supervisor/privacy lead and use an authorised channel. **Objective tested: 2–3.**
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Module 4 — Safeguarding and Protection from Harm

Lesson 10. Safeguarding Recognition and Risk

Why this matters in professional caregiving

Safeguarding is the work of preventing and stopping abuse, neglect and exploitation while promoting the person's wellbeing, voice and safety. It is not only a response to a confirmed incident. A caregiver may be the person who notices an unexplained injury, unusual fear, missing money, poor hygiene, a change in behaviour, a relative preventing private conversation or a colleague treating a resident harshly. Early recognition creates an opportunity to protect the client before harm increases.

Learning objectives

The learner should be able to describe safeguarding in practical terms; identify vulnerability and risk factors; distinguish a concern from proof; protect immediate safety; and report without investigating beyond role.

Core instructional content

Safeguarding means taking seriously the right of a person to live and receive care safely, free from abuse and neglect, while respecting their views, wishes and wellbeing. The WHO describes abuse of older people as a single or repeated act, or lack of appropriate action, within a relationship of expected trust that causes harm or distress. It includes physical, sexual, psychological or emotional abuse, financial or material abuse, abandonment, neglect and serious loss of dignity and respect ². SCIE similarly frames adult safeguarding around protecting the adult's right to live in safety and coordinated action by people and organisations ³. These references are international practice resources and are not Nigerian legal definitions.

Vulnerability is not a label that removes rights. A person may be more exposed to harm because of physical dependence, communication difficulty, disability, cognitive impairment, mental ill-health, poverty, isolation, illness, age, reliance on a particular caregiver or fear of losing housing or support. Risk can also arise from the environment, staffing, poor supervision, family conflict, financial dependency or organisational culture. A

person with a disability or an older person is not automatically helpless; the professional task is to provide support without treating vulnerability as incapacity.

A concern is information that reasonably makes a caregiver question safety or wellbeing. It may be a direct disclosure, an observation, a pattern, a third-party report or a mismatch between the care plan and what is happening. It is not the caregiver's role to decide whether abuse "really happened." Indicators can have innocent explanations, and abuse can be hidden. The safe response is to record facts, protect the person, report through the authorised pathway and allow qualified services to assess.

Immediate safety comes first. If there is current danger, serious injury, a medical emergency, a threat of violence or a risk that the alleged perpetrator will cause further harm, seek urgent assistance using the employer's emergency and safeguarding pathways. Do not confront a suspected abuser alone, remove evidence, threaten to call authorities or promise that the matter will stay secret. A caregiver should not place themselves or the client at greater risk in an attempt to "rescue" the situation.

Safeguarding is person-centred. Where safe and appropriate, ask what the client wants, what would help them feel safer and whom they trust. Do not let the client's wishes become an excuse to ignore immediate danger, but do not erase the client's voice either. A qualified safeguarding professional or manager decides how to balance autonomy, risk, confidentiality and reporting duties within the applicable framework.

Applying This in Practice

In **home care**, notice not only the client but also the household dynamics, access to food, money, medication and private communication. In **residential care**, look for patterns across shifts and residents. In **hospital-support settings**, follow the facility's safeguarding lead and incident pathway. In **community work**, report concerns even when the relationship is informal or the person is not receiving a formal institutional service.

Case scenario

A client who is usually talkative becomes quiet when a particular relative enters the room. The client has a new bruise and says, "Please do not make trouble."

Correct professional response: Maintain calm, create a safe opportunity for the client to speak privately if feasible, listen without leading, assess immediate safety, report to the designated supervisor/safeguarding route and document the exact observations and words. Do not confront the relative, promise secrecy or decide who caused the bruise.

Common professional risks

Risks include dismissing concerns as "family matters," assuming a bruise proves abuse, confronting an alleged abuser, asking leading questions, promising secrecy, waiting for

certainty, sharing rumours and failing to consider immediate danger.

Key practice points

1. Safeguarding protects safety and wellbeing while respecting the person's voice.
2. Vulnerability increases risk but does not remove rights.
3. A concern is not proof; report facts and patterns.
4. Protect immediate safety and do not investigate beyond role.
5. Never promise secrecy or confront a suspected perpetrator in a risky way.

Lesson assessment

1. **MCQ:** Safeguarding is best understood as: A) proving abuse; B) preventing and responding to risks of abuse, neglect and exploitation while promoting wellbeing; C) family discipline; D) a police-only activity. **Answer: B. Objective tested: 1.**
 2. **Scenario:** A client becomes fearful around one visitor. **Answer:** Observe, seek safe communication, protect immediate safety, report and document; do not accuse. **Objective tested: 2–5.**
 3. **True/False:** A concern must be proven before it is reported. **Answer: False. Objective tested: 3.**
 4. **What should the caregiver NOT do?** Confront the suspected abuser alone. **Answer:** It may increase risk and compromise the response. **Objective tested: 4.**
 5. **Best next action:** The client says, “Please keep this secret.” **Answer:** Listen respectfully but explain that information may need to be shared to keep them safe. **Objective tested: 4–5.**
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Lesson 11. Abuse, Neglect, Exploitation and Indicators

Why this matters in professional caregiving

Abuse and neglect may be obvious, subtle, intentional or the result of unsafe systems. A caregiver must recognise possible indicators without stereotyping families, accusing colleagues or confusing poverty with wrongdoing. The purpose of learning indicators is to notice change, ask appropriately, protect the client and trigger a qualified response.

Learning objectives

The learner should be able to distinguish common types of harm; identify indicators and risk factors; recognise grooming and manipulation as boundary concerns; and describe

what should and should not be done after noticing a concern.

Core instructional content

Physical abuse may involve hitting, pushing, burning, inappropriate restraint or rough handling. Indicators can include unexplained injuries, repeated falls with inconsistent explanations, fear of a particular person, flinching, injuries in different healing stages or a sudden change in mobility. These signs are not proof. A caregiver documents what is observed and seeks appropriate assessment.

Emotional or psychological abuse may involve threats, humiliation, insults, intimidation, isolation, controlling communication or treating an adult as a child. Indicators can include withdrawal, anxiety, unusual silence, sleep changes, loss of confidence or fear of speaking.

Sexual abuse includes any sexual contact, exposure, comment, image or act without valid consent. Warning signs may include distress after personal care, unexplained injuries, sexually transmitted infection concerns, fear or a disclosure. The caregiver must protect privacy and escalate urgently according to policy.

Financial or material abuse can include theft, coercion, misuse of bank cards or PINs, pressure to change a will, unauthorised borrowing, controlling pensions, taking benefits, or obtaining gifts through manipulation. Indicators may include unpaid essentials despite available funds, missing cash, unusual withdrawals, pressure from a relative, unexplained signatures or a caregiver showing unusual interest in the client's assets.

Neglect is a failure to provide necessary care or to act on known needs. It may be intentional or arise from poor staffing, inadequate training, lack of supplies or system failure. Indicators can include repeated missed meals, dehydration, untreated pain, poor hygiene, unsafe surroundings, missed appointments, pressure injuries, medication omissions or lack of appropriate clothing. A caregiver should consider both immediate harm and the system conditions contributing to it.

Discriminatory abuse may involve unequal treatment or harassment because of age, disability, sex, gender, religion, ethnicity, language, health condition or another characteristic. **Abandonment** includes leaving a person without necessary support or deliberately preventing access to help. **Inappropriate restraint or control** can involve tying, locking, threatening, unnecessary sedation, withholding communication devices or controlling money and visitors. **Self-neglect**, where relevant, involves a person's inability or unwillingness to meet essential needs; it calls for respectful assessment and support, not contempt.

Grooming and manipulation may appear as special attention, secrecy, gifts, isolating the client from others, creating emotional dependence, making the client feel indebted or gradually normalising requests for money, sexual contact, private meetings or rule-breaking. This can be perpetrated by a caregiver, family member, friend or another trusted

person. The caregiver should recognise patterns and report; not label a person or investigate motives.

Applying This in Practice

In **home care**, consider the interaction between financial vulnerability, family dependence and privacy. In **residential care**, compare information across shifts and residents and notice whether a worker treats one resident as a personal favourite. In **hospitals**, report unexplained injuries, discriminatory language or visitors who prevent safe communication. In **community settings**, pay attention to changes in access, housing, food and social connection.

Case scenario

A client says the caregiver should not tell anyone that a nephew takes the client's bank card because "he is the only family I have." The client has missed meals and cannot buy prescribed items.

Correct professional response: Listen without judgement, assess immediate safety and essential needs, do not confront the nephew or take the card, report through the safeguarding pathway, document the client's words and the observable consequences, and share information only with authorised people who need it to protect the client.

Common professional risks

Risks include stereotyping relatives, interpreting poverty as abuse without context, dismissing emotional abuse, treating financial abuse as a private family issue, using a label as if it were proof, and failing to report neglect caused by service systems.

Key practice points

1. Look for patterns and changes, not a single stereotype.
2. All forms of harm can be serious, including financial and emotional abuse.
3. System failure can produce neglect and must also be reported.
4. Grooming often develops through secrecy, special treatment and dependence.
5. Recognise, protect, report and document; do not investigate.

Lesson assessment

1. **Scenario:** A client has repeated unexplained injuries. **Answer:** Record objective facts, protect safety, seek appropriate assessment and report; do not decide guilt. **Objective tested: 2–3.**

2. **MCQ:** Which may indicate financial exploitation? A) a documented purchase requested by the client; B) pressure for PIN access and missing essentials; C) a normal receipt; D) a budget agreed with the client. **Answer: B. Objective tested: 2.**
 3. **True/False:** Emotional humiliation can be a safeguarding concern even without physical injury. **Answer: True. Objective tested: 2.**
 4. **What should the caregiver NOT do?** Take a client's bank card "for safekeeping" without authorised process. **Answer:** It creates a new financial and boundary risk. **Objective tested: 3–4.**
 5. **Scenario:** A colleague gives one client private gifts and asks for secrecy. **Answer:** Recognise possible grooming/boundary drift, document observable facts and report through the appropriate pathway; do not spread rumours. **Objective tested: 4–5.**
 6. **Best next action:** Several care tasks are repeatedly missed because staffing is inadequate. **Answer:** Protect immediate needs and report the service failure; neglect can result from system conditions. **Objective tested: 2–4.**
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Lesson 12. Responding to a Concern or Disclosure

Why this matters in professional caregiving

A client may disclose abuse directly, hint at it, deny it, protect the alleged perpetrator or change their account. A caregiver's first response can either increase safety or close the door to further help. The caregiver is not an investigator. The professional role is to listen calmly, protect immediate safety, use the authorised pathway and create an accurate record.

Learning objectives

The learner should be able to respond to a disclosure using safe communication; avoid secrecy promises and leading questions; protect immediate safety; report and escalate; and document the disclosure without embellishment.

Core instructional content

If a client discloses possible harm, remain calm and attentive. Thank them for telling you, acknowledge that the concern is important and ask only enough open questions to establish immediate safety and the information needed to report. Useful prompts include, "Tell me what happened," "What would help you feel safe right now?" and "Who else knows?" Avoid asking "Why did you let it happen?" or suggesting an answer.

Do not promise confidentiality in absolute terms. A safer explanation is: "I will respect your privacy, but I may need to share this with the people who can help keep you safe." Do

not promise a particular outcome, such as dismissal of a staff member, arrest or repayment of money. Do not confront the alleged perpetrator, mediate the conflict, search the person's belongings, interview witnesses or ask the client to repeat the story to many people.

Protect immediate safety within role. This may mean remaining with the client, moving to a safer space, obtaining medical or emergency assistance, preventing further access by an unsafe person through authorised measures, or calling a supervisor. Do not physically remove a person or restrain anyone unless an authorised emergency procedure and training apply. The caregiver's safety matters too.

Report promptly to the designated supervisor, safeguarding lead, manager, nurse, social worker, emergency contact or other authorised service specified by the employer. The pathway may differ depending on the client's age, immediate risk, type of concern and applicable jurisdiction. NIC should require every employer using the course to insert current reporting contacts before publication.

Document as soon as practical. Record the date, time, location, who was present, exact words where possible, observable condition, immediate actions, person informed and advice received. Mark uncertainty honestly. Do not rewrite the record to make it sound more dramatic or more certain. If a digital system is used, preserve the original entry according to policy.

After reporting, continue respectful care and follow up through the authorised route. Do not repeatedly question the client for updates or discuss the concern with colleagues who do not need to know. The client may feel ambivalent or withdraw; the caregiver should remain supportive while following the safety pathway.

Applying This in Practice

In **home care**, consider whether the alleged perpetrator is nearby and use the safest communication route. In **residential care**, notify the designated safeguarding lead and preserve the client's immediate safety. In **hospitals**, follow the hospital incident and safeguarding process. In **community work**, know how to obtain urgent support when away from a service base.

Case scenario

A client says, "My caregiver touches me in a way I do not like, but please do not tell anyone." The alleged caregiver is expected to return later that day.

Correct professional response: Listen calmly, thank the client, explain that safety information may need to be shared, assess immediate safety, prevent unsupervised contact through authorised arrangements, report urgently to the designated pathway and document the words and actions. Do not confront the alleged caregiver or promise secrecy.

Common professional risks

Risks include asking leading questions, promising secrecy, delaying the report, confronting the alleged person, investigating privately, recording opinions as facts, repeatedly questioning the client, spreading the disclosure and failing to make a safe plan for the next contact.

Key practice points

1. Listen, reassure and prioritise immediate safety.
2. Use open prompts; do not interrogate.
3. Never promise absolute secrecy.
4. Report through the authorised route without delay.
5. Document exact words, observations, actions and notifications.

Lesson assessment

1. **MCQ:** The best initial response to a disclosure is: A) “Why did you allow it?” ; B) “Thank you for telling me. What would help you feel safe now?” ; C) “I promise nobody will know” ; D) “I will confront them.” **Answer: B. Objective tested: 1.**
2. **Scenario:** An alleged perpetrator is due to arrive. **Answer:** Protect immediate safety, arrange authorised separation, escalate urgently and document. **Objective tested: 3–4.**
3. **True/False:** The caregiver should investigate to find out whether the client is telling the truth. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Promise that the disclosure will remain secret. **Answer:** Explain limited, safety-related information sharing. **Objective tested: 2.**
5. **Documentation question:** What should be recorded? **Answer:** Exact words where possible, date/time, observations, actions, notifications and advice. **Objective tested: 5.**
6. **Best next action:** The client withdraws the disclosure later. **Answer:** Do not pressure or interrogate; report the change and follow the authorised safeguarding process. **Objective tested: 3–4.**
7. **Scenario:** A colleague asks for details “to help investigate.” **Answer:** Share only through the authorised process with people who have a legitimate role; do not discuss informally. **Objective tested: 4–5.**

Module 5 — Professional Boundaries, Gifts and Conflicts

Lesson 13. Boundary Drift and Relationship Risks

Why this matters in professional caregiving

Professional boundaries protect the client, caregiver and integrity of the service. Boundary drift often begins with a small exception: a private message, a favour, a personal secret, a ride home, a visit outside work or a gift accepted “just this once.” Not every warm relationship is unsafe, but a pattern of secrecy, dependency, preferential treatment or personal benefit can create safeguarding and exploitation risks.

Learning objectives

The learner should be able to identify early boundary crossings; distinguish warmth from inappropriate dual relationships; manage emotional dependence and private contact; and seek supervision before a small crossing becomes a larger risk.

Core instructional content

A professional relationship has a purpose, role, time, place and accountability structure. Boundaries define what belongs within that relationship and what should remain outside it. A caregiver can be compassionate, friendly and culturally respectful without becoming the client’s friend, business partner, borrower, romantic partner or secret-keeper.

Boundary drift is a gradual movement away from the professional role. Indicators include thinking about one client constantly, feeling that only you understand them, hiding contact from colleagues, accepting special treatment, sharing increasingly personal information, meeting outside work, using client property, doing extra unpaid tasks for one client, communicating late at night or becoming defensive when questioned. These are prompts to pause, not proof of misconduct.

Emotional dependence can develop on either side. A caregiver may use the client to meet their own need for companionship, validation, money or family connection. A client may feel abandoned if the caregiver takes leave, creating pressure for private contact. The professional response is to acknowledge the relationship respectfully, maintain consistent limits, discuss concerns with a supervisor and ensure continuity through the care team.

Romantic or sexual relationships with current clients are incompatible with the professional caregiving role because of the power imbalance and dependency. Sexual comments, flirtation, intimate messages, unnecessary touch and requests for private meetings are boundary risks. If a client makes an advance, the caregiver should respond clearly and respectfully, redirect to the professional relationship and inform a supervisor if the situation continues or affects safe care.

Personal disclosure should be purposeful and limited. A small, relevant disclosure may build rapport, but the caregiver should not tell a client about financial problems, marital

conflict, trauma, attraction, political arguments or private plans in a way that invites the client to comfort or rescue them. The test is: **Does this disclosure serve the client's care, or does it recruit the client into my personal life?**

Private visits, transport and use of client property require clear policy. Do not visit after hours, accept invitations to family events, use a client's vehicle or borrow belongings without authorisation. If an exceptional practical need arises, consult the supervisor and document the arrangement.

Applying This in Practice

In **home care**, familiarity and hospitality can blur role; use transparent boundaries and explain them kindly. In **residential care**, ensure no resident becomes a staff member's "special person." In **hospitals**, keep communication within approved channels. In **community work**, avoid private social relationships that cannot be supervised.

Case scenario

A client sends a caregiver a personal message every evening and says, "You are the only person who really cares. Please do not tell your supervisor." The caregiver feels valued and begins replying after work.

Correct professional response: Recognise emotional dependence and secrecy as boundary risks, stop using the private channel, communicate through the approved route, discuss the situation with the supervisor and document relevant professional concerns. The caregiver should not continue in secret or punish the client for expressing attachment.

Common professional risks

Risks include secrecy, special treatment, personal disclosure, private messaging, after-hours visits, romantic or sexual involvement, using client property, accepting invitations and believing that good intentions remove power imbalance.

Key practice points

1. Small crossings can become larger professional risks.
2. Warmth is compatible with clear limits.
3. Secrecy and emotional dependence require early supervision.
4. Never pursue romantic or sexual relationships with current clients.
5. Transparency protects both the client and caregiver.

Lesson assessment

1. **MCQ:** Boundary drift often begins with: A) a clearly documented care task; B) a small exception that becomes a pattern; C) a team handover; D) a routine greeting. **Answer: B. Objective tested: 1.**
 2. **Scenario:** A client says the caregiver is the only person who cares and asks for secrecy. **Answer:** Acknowledge, maintain the professional channel, notify the supervisor and document. **Objective tested: 2–3.**
 3. **True/False:** Good intentions eliminate the power imbalance between caregiver and client. **Answer: False. Objective tested: 2.**
 4. **What should the caregiver NOT do?** Continue a secret romantic or emotionally dependent relationship. **Answer:** It creates serious boundary and safeguarding risk. **Objective tested: 3–4.**
 5. **Best next action:** The caregiver notices they are thinking about one client constantly. **Answer:** Pause, seek supervision and review boundaries and workload. **Objective tested: 1–3.**
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Lesson 14. Gifts, Money and Financial Safeguards

Why this matters in professional caregiving

Money and gifts are especially sensitive in home and community care. A client may offer cash out of gratitude, request a loan, promise an inheritance, ask the caregiver to use a bank card or place the caregiver in a will. Even when the client appears willing, the caregiver's position of trust can make the exchange exploitative or create suspicion, dependency and conflict. The principle is simple: **Do not use a position of trust for personal financial benefit.**

Learning objectives

The learner should be able to identify financial boundary risks; respond to gifts and offers without humiliation; handle client money only through authorised processes; protect bank cards and PINs; and report suspected exploitation.

Core instructional content

Employer rules vary. Some organisations prohibit all gifts; others allow low-value tokens subject to disclosure and a register. The caregiver must follow the current policy rather than create a personal threshold. Cash, expensive gifts, jewellery, electronics, loans, tips, discounts, property, inheritance promises and being named in a will should be treated as high-risk and disclosed or declined according to policy.

When a client offers a small gift, respond respectfully: “Thank you for thinking of me. I need to follow the organisation’s gift policy, so I will check before accepting anything.” If the policy prohibits acceptance, decline without shaming: “Your thanks are meaningful, but I cannot accept money or personal gifts in my role.” Offer an approved alternative such as a note of appreciation or feedback to the supervisor.

Never borrow from or lend money to a client. Do not receive a loan, co-sign a loan, recommend a personal investment, sell products, solicit donations, ask for an inheritance or encourage a client to change a will. Do not accept a client’s bank card, PIN, online password or personal financial credentials. If shopping is part of the role, use the authorised process, obtain receipts, keep client funds separate and return change promptly.

Handling client cash requires transparent controls. Follow the care plan and employer procedure, use a transaction record where required, obtain a signature or other approved confirmation, and report discrepancies immediately. Do not use client cash temporarily, combine it with personal money, buy an unapproved item or say “I will remember the amount.”

Financial exploitation may be perpetrated by relatives, friends, staff or outsiders. Indicators include unexplained withdrawals, missing essentials, pressure to give money, sudden changes to documents, a caregiver’s unusual interest in assets, or a client expressing fear about finances. The caregiver should not take control of the client’s money to “protect” them. Report, preserve records and seek authorised safeguarding support.

Applying This in Practice

In **home care**, family hospitality and gift-giving may be customary; explain policy respectfully and disclose offers. In **residential care**, use gift registers and team transparency. In **hospitals**, do not handle money or cards unless an authorised process exists. In **community settings**, keep client and personal funds completely separate.

Case scenario

A client offers the caregiver an expensive phone and says, “It will be yours when I die; do not tell the office.”

Correct professional response: Decline or place the offer on hold under the gift policy, state that secrecy is not appropriate, inform the supervisor and document the offer. The caregiver should not accept, negotiate, promise future benefit or discuss changing a will.

Common professional risks

Risks include cash tips, loans, inheritance promises, use of PINs, unreceipted shopping, mixing funds, personal sales, “temporary” borrowing, accepting expensive gifts and concealing transactions.

Key practice points

1. Do not use a position of trust for personal financial benefit.
2. Follow employer gift and money-handling policy.
3. Never use a client's PIN or borrow their money.
4. Keep transparent records for authorised purchases.
5. Report suspected exploitation without taking over the client's assets.

Lesson assessment

1. **MCQ:** The safest response to an expensive gift is: A) accept secretly; B) check policy, decline or disclose appropriately; C) ask for cash instead; D) promise to repay later. **Answer: B. Objective tested: 1–2.**
 2. **Scenario:** A client asks the caregiver to use a bank card and PIN for shopping. **Answer:** Follow the authorised money-handling process; never use personal possession of the PIN or card outside it, and escalate uncertainty. **Objective tested: 3–4.**
 3. **True/False:** A client's gratitude makes it acceptable to borrow money. **Answer: False. Objective tested: 1.**
 4. **What should the caregiver NOT do?** Keep cash temporarily and rely on memory. **Answer:** Use the authorised record and receipt process. **Objective tested: 3.**
 5. **Scenario:** A client promises an inheritance. **Answer:** Do not discuss or encourage it; report the boundary concern and maintain professional distance. **Objective tested: 1–2.**
 6. **Best next action:** Money is missing from an authorised shopping transaction. **Answer:** Preserve the record, report immediately and do not replace or conceal the amount personally. **Objective tested: 4–5.**
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Lesson 15. Conflicts of Interest and Preferential Treatment

Why this matters in professional caregiving

A conflict of interest arises when a caregiver's personal, financial or relational interest could influence—or appear to influence—professional judgement. A conflict does not require proven wrongdoing. Early disclosure allows a supervisor to manage the risk. Favouritism can harm other clients, create dependency and undermine confidence in the service.

Learning objectives

The learner should be able to identify actual, potential and perceived conflicts; disclose them early; avoid recommending products or services for personal gain; and provide consistent, client-centred care.

Core instructional content

An **actual conflict** exists when personal interests directly conflict with the client's interests. A **potential conflict** could develop if circumstances change. A **perceived conflict** may reasonably cause others to question impartiality. All three deserve attention. For example, a caregiver may be related to a client, sell mobility products, have a personal relationship with a family member or receive a referral benefit.

Conflicts can involve money, family, friendships, gifts, business, transport, property, employment, social groups or personal beliefs. A caregiver should not recommend a product, agency, church, school, loan service or supplier because they receive a commission or favour. If a client asks for a recommendation, refer to the organisation's approved information or disclose the conflict and seek direction.

Favouritism includes giving one client extra time, better food, preferred shifts, private access, gifts, faster service or relaxed rules without a care-based reason. It can also mean treating a disliked client less patiently. Professional equality does not mean identical care; a client may appropriately receive different support according to need. The difference should be transparent, care-related and authorised.

Use disclosure as a control. Tell the supervisor before accepting an assignment or as soon as a conflict arises. Do not wait until a complaint occurs. The supervisor may reassign the caregiver, set safeguards, document the decision or permit the work with controls. The caregiver should not decide alone that a conflict is harmless.

The client's interests remain central. If a caregiver's business, family or emotional interests influence a decision, the caregiver should pause. Ask: "Would I make the same recommendation if I received no personal benefit? Would I be comfortable if the supervisor and client's advocate read this record?" If the answer is no or uncertain, disclose and seek guidance.

Applying This in Practice

In **home care**, relationships within close communities may overlap; transparency is essential. In **residential care**, avoid private "favourite" arrangements and undocumented exceptions. In **hospitals**, use approved referral and supplier information. In **community settings**, do not exploit local trust to sell services or obtain clients for personal business.

Case scenario

A caregiver's cousin owns a home-care agency. The client asks whether to move to that agency, and the caregiver would receive a referral payment.

Correct professional response: Disclose the conflict, do not pressure or recommend the cousin's agency for personal benefit, provide only approved neutral information and refer the client to the authorised decision-maker or process.

Common professional risks

Risks include personal commissions, family assignments, selling products, using client resources, giving special treatment, hiding conflicts, treating a client differently because of personal dislike and making undocumented exceptions.

Key practice points

1. Actual, potential and perceived conflicts should be disclosed.
2. Personal benefit must not influence care decisions.
3. Different care can be fair when based on assessed need and authorised planning.
4. Transparency is safer than private judgement.
5. Do not use a trusted role to recruit business.

Lesson assessment

1. **MCQ:** A perceived conflict matters because: A) only proven misconduct matters; B) trust can be damaged even when impartiality is questioned; C) it is always a crime; D) it requires secrecy. **Answer: B. Objective tested: 1.**
2. **Scenario:** The caregiver may receive a referral payment. **Answer:** Disclose it, avoid personal recommendation and use the approved neutral process. **Objective tested: 2–3.**
3. **True/False:** Favouritism is acceptable if the favoured client is grateful. **Answer: False. Objective tested: 3.**
4. **What should the caregiver NOT do?** Hide a family relationship with a client. **Answer:** Disclose and seek management. **Objective tested: 2.**
5. **Best next action:** A caregiver realises personal dislike is affecting patience. **Answer:** Seek supervision, reflect and ensure care remains consistent and respectful. **Objective tested: 3–4.**
6. **Scenario:** A client asks for a product recommendation. **Answer:** Use approved information, disclose any conflict and do not sell or profit through the role. **Objective tested: 2.**

Module 6 — Professional Conduct, Reporting and Accountability

Lesson 16. Conduct, Reliability and Honest Practice

Why this matters in professional caregiving

Professional conduct is the daily evidence that a caregiver can be trusted. Punctuality, reliable attendance, respectful language, appropriate appearance, accurate handover and honest documentation affect safety. A missed visit, late arrival, unreported absence, falsified record or concealed medication-related incident can place a client at risk even when the caregiver meant well.

Learning objectives

The learner should be able to connect reliability to client safety; report lateness and missed care; respond to mistakes honestly; challenge falsification; and distinguish acceptable explanation from excuse-making.

Core instructional content

Punctuality matters because clients may depend on meals, toileting, medication support, transfers, appointments or supervision at a particular time. Unexpected lateness can happen. Professional conduct requires notifying the service as soon as possible, protecting urgent needs, handing over safely and recording what changed. A caregiver should not switch off a phone, ask a client to conceal lateness or mark attendance inaccurately.

Following the care plan is a professional duty within scope. If a plan seems unsafe, outdated, impossible or inconsistent with the client's current condition, raise it. Do not silently substitute a personal routine. Handover should include changes, refusals, incidents, supplies, family requests and pending actions.

Honesty after an error protects the client. If a task was missed, a record is wrong, a medication-support step was delayed or a boundary was crossed, report promptly through the appropriate incident route. Do not destroy notes, edit a record to hide the original, persuade a colleague to stay silent or blame the client. A fair learning culture distinguishes human error from reckless conduct, but both require truthful information.

If a supervisor or colleague asks the caregiver to falsify or conceal an incident, the caregiver should refuse respectfully and use the next appropriate reporting route. The caregiver should state, "I cannot record that care was completed when it was not. I will document what happened and seek guidance." If the usual supervisor is involved in the concern, the employer's alternative escalation route should be used.

Appearance and language should support dignity and safety. Follow uniform, identification, hygiene and footwear rules. Avoid shouting, sarcasm, nicknames the client dislikes, discriminatory jokes and conversations that expose private matters. Professional warmth includes listening, explaining delays and apologising appropriately without making the client responsible for comforting the caregiver.

Applying This in Practice

In **home care**, communicate travel delays and protect continuity. In **residential care**, complete handover and incident records before leaving. In **hospitals**, follow attendance, medication-support and documentation systems. In **community work**, plan travel and keep client information secure while reporting delays.

Case scenario

A colleague asks the caregiver to sign that a repositioning task was completed because the colleague forgot it. The client now has redness on the skin.

Correct professional response: Do not sign falsely. Assess and report the client's condition within scope, notify the responsible nurse/supervisor, document the facts and follow the incident pathway. The caregiver should not accuse the colleague publicly or alter the record.

Common professional risks

Risks include falsifying records, concealing incidents, silent lateness, missed handover, disrespectful language, blaming clients, ignoring care-plan changes and accepting unsafe instructions from senior staff.

Key practice points

1. Reliability is part of client safety.
2. Report delays and missed care promptly.
3. Never falsify or conceal a record.
4. Refuse unsafe or dishonest instructions respectfully.
5. Use authorised escalation when a supervisor is involved.

Lesson assessment

1. **MCQ:** If late, the caregiver should: A) hide it; B) notify the service, protect urgent needs and document; C) ask the client to lie; D) mark on time. **Answer: B. Objective tested: 1–2.**

2. **Scenario:** A colleague asks for a false signature. **Answer:** Refuse, report the actual facts and escalate the client's condition. **Objective tested: 3–4.**
 3. **True/False:** Good intentions justify falsifying a care record. **Answer: False. Objective tested: 3.**
 4. **What should the caregiver NOT do?** Edit a record to erase a missed task. **Answer:** Use the approved correction process and report. **Objective tested: 3.**
 5. **Best next action:** The care plan no longer appears safe. **Answer:** Pause the unsafe element, provide only safe support within scope and seek qualified review. **Objective tested: 2.**
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Lesson 17. Reporting, Escalation and Documentation

Why this matters in professional caregiving

A concern becomes safer when it reaches the right person in time. Reporting is not betrayal, disloyalty or unnecessary troublemaking. It is a professional action that protects clients, supports accountability and allows a service to respond. The quality of the report matters: it should be clear enough to support action without exaggeration, gossip or unsupported conclusions.

Learning objectives

The learner should be able to choose between urgent escalation and routine reporting; identify the appropriate recipient; give a concise factual handover; document objectively; and follow up without breaching confidentiality.

Core instructional content

Use the level of urgency to choose the channel. Immediate danger, serious injury, sexual abuse concern, active violence, disappearance, medical emergency or imminent financial exploitation requires immediate verbal or emergency escalation according to employer and public-authority pathways. A non-urgent boundary concern, repeated disrespect or a policy gap may be reported through a supervisor, incident form or scheduled safeguarding review. When unsure, escalate sooner and explain the uncertainty.

A report should include the situation, relevant background, objective observations, immediate risk, actions taken, who is involved, what the client said if relevant, and what assistance is requested. Avoid labels such as “bad family” or “definitely abusive.” Use “client reported...,” “I observed...,” “the record shows...” and “I am concerned because...” .

Document promptly, securely and separately from informal notes. Record date, time, location, task, people present, exact words where possible, visible condition, action taken, recipient and advice. If the record is corrected, follow the authorised audit trail. Do not backdate, delete, rewrite history or use a personal notebook as the permanent record unless the organisation authorises it.

Confidentiality and safeguarding must be balanced. Information should be shared with people who need it to protect the client or manage the concern. Do not tell unrelated colleagues, family members, community leaders or online groups. If a client asks who will be told, explain honestly within role and refer detailed questions to the safeguarding lead.

Follow-up means confirming that the report was received, carrying out agreed actions, observing the client, attending review and reporting any escalation or retaliation. It does not mean conducting a private investigation. If a concern is not addressed, use the next authorised route. If the supervisor is implicated, use the alternative pathway specified by the employer.

NIC should require portal implementation teams to add local contacts: supervisor, designated safeguarding lead, emergency service, occupational-health or clinical lead, privacy/data-protection contact, complaints route and any relevant state or facility reporting pathway. The course must not invent or imply one universal Nigerian reporting route.

Applying This in Practice

In **home care**, know whom to call outside office hours. In **residential care**, use handover and incident systems. In **hospitals**, follow safeguarding, complaints and clinical escalation routes. In **community care**, carry the service contact and do not rely on the client's relative to pass on an urgent concern.

Case scenario

A caregiver reports that a supervisor asked them to conceal a client's fall. The supervisor tells the caregiver not to "make trouble."

Correct professional response: Protect the client and seek clinical review as required, preserve objective facts, refuse concealment and use the employer's alternative escalation route for concerns involving the supervisor. Document the request and response confidentially.

Common professional risks

Risks include reporting only to the alleged perpetrator, delaying urgent reports, using gossip, failing to record exact words, backdating, sharing screenshots, not escalating when the first route fails and assuming a form has been read.

Key practice points

1. Match urgency to the reporting channel.
2. Report facts, not conclusions or rumours.
3. Document time, words, observations, actions and notifications.
4. Use an alternative route when the usual person is involved.
5. Follow up without investigating or oversharing.

Lesson assessment

1. **MCQ:** Imminent danger requires: A) next week's meeting; B) immediate authorised escalation; C) a social-media post; D) silence. **Answer: B. Objective tested: 1.**
 2. **Scenario:** The usual supervisor is implicated. **Answer:** Use the alternative authorised pathway and document the concern. **Objective tested: 2–4.**
 3. **True/False:** “Family is abusive” is sufficient objective documentation. **Answer: False. Objective tested: 3.**
 4. **What should the caregiver NOT do?** Assume a completed form has been read when the risk is urgent. **Answer:** Confirm direct receipt through the required channel. **Objective tested: 1 and 5.**
 5. **Scenario:** Give a concise report for a client disclosure. **Answer:** State time/place, exact words, immediate safety, actions, person informed and requested follow-up. **Objective tested: 2–3.**
 6. **Best next action:** No response follows a serious report. **Answer:** Escalate through the next authorised route and document the follow-up. **Objective tested: 5.**
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Lesson 18. Integrated Ethical Decision-Making

Why this matters in professional caregiving

Complex situations rarely fit one category. A client may offer money while disclosing abuse; a family member may demand information while the client refuses care; a caregiver may feel emotionally attached while noticing a colleague's neglect. The professional standard is the ability to recognise the issue, protect rights and safety, act within role, report and document without losing compassion or confidentiality.

Learning objectives

The learner should be able to use NIC SAFE-CARE in a complex scenario; prioritise immediate safety; balance autonomy and duty; distinguish professional action from investigation; and produce a defensible objective record.

Core instructional content

Start by naming the concern without prematurely deciding what it means. Is the issue a boundary, privacy breach, consent concern, safeguarding risk, conflict of interest, conduct problem or combination? Naming the issue helps the caregiver avoid normalising it.

Next, pause and assess immediate safety. Is someone in danger now? Is urgent medical or emergency help required? Is the client alone with a person who may cause harm? Has a record, bank card or image been exposed? Is the caregiver being asked to perform an unsafe or unauthorised act? Immediate safety takes priority over routine schedules and over completing a form.

Consider rights and professional duties together. Ask what the client wants, what dignity requires, what consent permits, what confidentiality protects, what duty of care requires and what the care plan or policy says. A client's preference matters, but a caregiver should not promise secrecy if safety information must be shared. A family's concern matters, but it does not automatically replace the client's voice. A caregiver's personal belief matters as a reflection point, but it cannot justify discrimination or coercion.

Check scope. The caregiver can listen, provide basic safe support, maintain privacy, refuse an inappropriate request, protect immediate safety within training, report, escalate and document. The caregiver cannot diagnose, make formal legal capacity determinations, investigate abuse, adjudicate guilt, prescribe, change a care plan independently, manage client assets for personal benefit or promise an outcome.

Communicate with disciplined language. Use clear sentences, exact words, relevant observations and a direct request. Avoid labels, emotional speculation and unnecessary detail. After reporting, maintain confidentiality and follow up through the authorised process. If the concern involves a supervisor or a usual reporting route, use the alternative pathway.

A final reflective test is useful: **Would I be comfortable explaining this action to the client, supervisor, safeguarding lead and an impartial reviewer?** If not, pause and seek guidance. Transparency does not guarantee that every decision will be easy, but it reduces hidden risk.

Applying This in Practice

In **home care**, use the framework when family, money and privacy overlap. In **residential care**, use it at handover, during complaints and after incidents. In **hospitals**, apply it to

delegated care and clinical escalation. In **community settings**, use it when relationships and professional roles overlap.

Case scenario

A client privately tells a caregiver that a relative takes money, then offers the caregiver cash to keep the conversation secret. The relative later asks the caregiver for a photograph of the client and says, “You work for us, so you must tell me everything.”

Correct professional response: Recognise financial safeguarding, gift/boundary and confidentiality issues; assess immediate safety; do not accept the cash; explain that safety concerns cannot be kept secret; do not send a photograph or disclose more than authorised; report through the safeguarding and supervisory pathway; document exact words, observations, actions and notifications; and maintain confidentiality. Do not confront the relative or investigate.

Common professional risks

Risks include choosing the easiest person to satisfy, accepting money to preserve trust, disclosing to family without authorisation, delaying escalation, investigating privately, documenting conclusions and allowing emotional discomfort to determine action.

Key practice points

1. Complex situations require a structured pause.
2. Rights, safety and professional duty must be considered together.
3. Act within role and escalate beyond it.
4. Do not let money, fear, family pressure or attachment distort judgement.
5. A clear, confidential, objective record is part of the response.

Lesson assessment

1. **Integrated scenario:** The client offers cash after disclosing possible abuse. What is the first professional response? **Answer:** Protect immediate safety, decline the cash, listen calmly and explain that safety information may need to be shared. **Objective tested: 1–3.**
2. **MCQ:** The caregiver’s strongest control is: A) secrecy; B) SAFE-CARE; C) personal investigation; D) public accusation. **Answer: B. Objective tested: 1.**
3. **True/False:** Family pressure can justify sharing all client information. **Answer: False. Objective tested: 2–4.**

4. **What should the caregiver NOT do?** Confront the relative before notifying the safeguarding pathway. **Answer:** It may increase risk or compromise the response. **Objective tested: 3–4.**
 5. **Documentation question:** What makes the record defensible? **Answer:** Timely facts, exact words where possible, observations, actions, notifications and follow-up without unsupported conclusions. **Objective tested: 5.**
 6. **Best next action:** The caregiver is unsure whether a concern is “serious enough.” **Answer:** Pause, protect, seek guidance and report the concern with facts rather than waiting for certainty. **Objective tested: 1–4.**
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13. Lesson assessments

Lesson assessments are embedded after each lesson. NIC should randomise equivalent items while keeping the professional reasoning visible. A recommended lesson threshold is 70–80% with immediate explanations and an opportunity to review. Lesson assessments are formative unless NIC chooses to make them progression requirements.

14. Module assessments

Each module assessment should contain 5–8 questions and test application rather than definition recall. Recommended threshold: 80%, with one retry after review.

Module 1 assessment: Professional values, dignity and ethical judgement

1. **Scenario:** A family asks the caregiver to ignore a client’s stated preference because it is inconvenient. **Answer:** Respect the client’s preference where safe, explain professional duty and seek guidance if safety is affected. **Explanation:** Family involvement is valuable but does not automatically replace the client’s voice. **LO:** 1–3.
2. **MCQ:** Which is a professional duty? A) hiding mistakes; B) honesty, dignity and accountability; C) imposing beliefs; D) rewarding compliance. **Answer:** B. **LO:** 1.
3. **Scenario:** A client is exposed during bathing because supplies were not prepared. **Answer:** Cover the client, complete care respectfully, review the workflow and report recurring risk. **Explanation:** Dignity is protected through preparation and privacy. **LO:** 1 and 8.
4. **True/False:** A cultural practice can justify humiliating a client. **Answer:** False. **LO:** 2–3.

5. **Best next action:** A caregiver realises their personal belief is affecting care. **Answer:** Pause, seek supervision and ensure the client receives respectful, non-discriminatory care. **LO:** 2.
6. **Scenario:** A missed task is discovered at handover. **Answer:** Report, document accurately, arrange safe follow-up and do not falsify the record. **LO:** 8.

Module 2 assessment: Client rights, consent and autonomy

1. **Scenario:** A client refuses intimate care and says they are frightened. **Answer:** Pause, listen, offer safe alternatives and escalate the concern; do not force. **Explanation:** Refusal is meaningful information and may indicate pain, fear or safeguarding risk. **LO:** 4–5.
2. **MCQ:** Consent is: A) permanent; B) voluntary, task-specific and withdrawable; C) supplied by any relative; D) unnecessary for routine care. **Answer: B. LO:** 4.
3. **Scenario:** A family member insists on deciding clothing for a client who can communicate a preference. **Answer:** Centre the client's choice, support communication and seek guidance only if genuine safety or decision-support concerns arise. **LO:** 5.
4. **True/False:** Caregivers may declare a client legally incapable because communication is slow. **Answer: False. LO:** 5.
5. **Scenario:** A facility leaves bathroom doors open for convenience. **Answer:** Raise the privacy risk and seek the least-restrictive safe alternative. **LO:** 6.
6. **What should the caregiver NOT do?** Threaten or shame a client into accepting care. **Answer:** Coercion undermines consent and dignity. **LO:** 1 and 4.

Module 3 assessment: Confidentiality, privacy and digital professionalism

1. **Scenario:** A family member requests daily medical updates to a personal number. **Answer:** Check authorisation and use approved channels; do not begin private messaging. **LO:** 6.
2. **MCQ:** Minimum-necessary disclosure means: A) all details to everyone; B) only relevant information to authorised people; C) no information ever; D) anonymous posting. **Answer: B. LO:** 6.
3. **Scenario:** A wound photo is on a personal phone. **Answer:** Do not forward it; report immediately and follow the approved privacy/clinical process. **LO:** 6.

4. **True/False:** Removing a name from a post always protects anonymity. **Answer: False.** **LO:** 6.
5. **Scenario:** A screenshot is sent to the wrong group. **Answer:** Report promptly, preserve evidence as instructed and do not try to conceal it. **LO:** 6 and 8.
6. **What should the caregiver NOT do?** Argue publicly about a complaint using client details. **Answer:** Use the authorised complaint route. **LO:** 6 and 8.

Module 4 assessment: Safeguarding and protection from harm

1. **Scenario:** The client says a relative takes money and asks for secrecy. **Answer:** Listen, explain limits of confidentiality, assess safety, report and document. **LO:** 3–4.
2. **MCQ:** A safeguarding concern is: A) proof of guilt; B) information requiring safe attention and appropriate reporting; C) gossip; D) a caregiver investigation. **Answer: B.** **LO:** 3.
3. **Scenario:** A client discloses sexual touching. **Answer:** Stay calm, listen, protect immediate safety, report urgently and document exact words; do not confront or investigate. **LO:** 4.
4. **True/False:** Neglect can result from system failure as well as individual action. **Answer: True.** **LO:** 3.
5. **What should the caregiver NOT do?** Ask the client to repeat the disclosure to multiple colleagues. **Answer:** Share only through the authorised pathway. **LO:** 4.
6. **Scenario:** A colleague gives a resident private gifts and asks for secrecy. **Answer:** Recognise grooming/boundary risk, document facts and report. **LO:** 3 and 4.
7. **Best next action:** The suspected perpetrator is expected to return shortly. **Answer:** Make a safe plan and escalate urgently. **LO:** 4.

Module 5 assessment: Professional boundaries, gifts and conflicts

1. **Scenario:** A client offers an expensive gift. **Answer:** Follow policy, decline or disclose appropriately and do not keep it secret. **LO:** 7.
2. **MCQ:** Which is a high-risk boundary? A) approved handover; B) borrowing money from a client; C) knocking before entering; D) using a care plan. **Answer: B.** **LO:** 7.
3. **Scenario:** The caregiver receives a friend request from a client. **Answer:** Do not accept through the personal account; maintain approved professional communication. **LO:** 7.

4. **True/False:** A caregiver may enter a client's will if the client insists. **Answer: False.** It creates a serious conflict and exploitation risk. **LO: 7.**
5. **Scenario:** The caregiver can earn a referral payment from a relative's agency. **Answer:** Disclose, avoid personal recommendation and use neutral approved information. **LO: 7.**
6. **Best next action:** One client consistently receives undocumented extra privileges. **Answer:** Review the care-related reason, disclose the pattern and correct favouritism through supervision. **LO: 7–8.**

Module 6 assessment: Conduct, reporting and accountability

1. **Scenario:** A supervisor asks the caregiver to conceal a fall. **Answer:** Protect the client, refuse concealment, document and use the alternative escalation route. **LO: 8–9.**
2. **MCQ:** An urgent risk should be: A) placed in a queue; B) escalated immediately through the authorised channel; C) discussed online; D) ignored. **Answer: B. LO: 4 and 9.**
3. **True/False:** A report should contain facts and observations rather than unsupported conclusions. **Answer: True. LO: 4 and 9.**
4. **Scenario:** A concern is not addressed after the first report. **Answer:** Follow up and use the next authorised route; document the follow-up. **LO: 9.**
5. **Scenario:** A colleague asks for a false signature. **Answer:** Refuse, report accurately and escalate any client risk. **LO: 8.**
6. **Integrated question:** A client discloses financial abuse, offers money, and a relative requests a photograph. **Answer:** Decline money, protect safety, do not share the photograph, report, document and maintain confidentiality. **LO: 3–9.**
7. **Best next action:** Unsure whether a concern is serious enough. **Answer:** Pause, protect, seek guidance and report facts rather than waiting for certainty. **LO: 3–4 and 9.**

15. Final assessment blueprint

The final assessment contains **35 questions** and should require **80% (28/35)** to pass. At least **23 questions are scenario/application based**. The suggested distribution follows the course journey and ensures that ethical judgement, safeguarding, boundaries, consent, confidentiality, reporting, documentation and scope are assessed.

Questions	Content area	Number	Application/scenario target
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1–5	Professionalism, ethics and dignity	5	4
6–10	Consent, autonomy and client rights	5	4
11–16	Confidentiality, privacy and digital professionalism	6	5
17–24	Safeguarding and abuse recognition	8	6
25–29	Boundaries, gifts and financial issues	5	3
30–35	Integrated judgement, reporting, documentation and escalation	6	5
Total		35	27

16. Final assessment questions

- Scenario:** A caregiver is late and discovers that a client's morning care was missed. The caregiver is tempted to mark it as completed. What is the best action?

A) Mark it complete to avoid criticism. B) Notify the service, provide safe care within the remaining time, document what occurred and arrange follow-up. C) Ask the client to say it was completed. D) Ignore it.

Correct answer: B. This protects the client and preserves honesty. **LO:** 1 and 8.
- Scenario:** A caregiver's personal belief conflicts with a client's harmless clothing preference. What should the caregiver do?

A) Refuse the choice. B) Mock it. C) Respect the preference and seek support if a genuine role or safety issue exists. D) Ask the family to punish the client.

Correct answer: C. Professional responsibility requires respect and non-discrimination. **LO:** 2.
- MCQ:** Which action best demonstrates dignity during intimate care?

A) Expose the whole body to work faster. B) Explain, ask permission, close the door and expose only what is necessary. C) Discuss the client with visitors. D) Use a nickname

without asking.

Correct answer: B. LO: 1.

4. **Scenario:** A colleague asks the caregiver to sign for a task that was not completed. What should the caregiver do?

A) Sign because the colleague is senior. B) Refuse, record accurately and report the client risk. C) Sign and edit later. D) Say nothing.

Correct answer: B. LO: 8.

5. **True/False:** Professional warmth requires a caregiver to become a client's personal friend.

Correct answer: False. Warmth and boundaries can coexist. **LO: 1 and 7.**

6. **Scenario:** A client refuses bathing because of pain. A relative says to force the client. What is the best next action?

A) Force the bath. B) Respect the refusal, explore the pain within role, offer safe alternatives and escalate. C) Threaten the client. D) Hide the refusal.

Correct answer: B. LO: 4 and 5.

7. **MCQ:** Consent is best understood as:

A) permanent permission; B) voluntary and task-specific agreement that can be withdrawn; C) permission from any relative; D) unnecessary for routine care.

Correct answer: B. LO: 4.

8. **Scenario:** A family member demands that a client not be told about a planned change in care, but the client asks what will happen. What should the caregiver do?

A) Lie. B) Reveal all family information. C) Avoid lying, refer the question to the responsible professional and document the request. D) Tell the client the family is controlling.

Correct answer: C. LO: 5.

9. **Scenario:** A client communicates slowly but chooses their clothing. What should the caregiver do?

A) Let a relative choose automatically. B) Allow time and support the client's choice. C) Ignore the client. D) Declare incapacity.

Correct answer: B. LO: 5.

10. **What should the caregiver NOT do?** Use threats, humiliation or withholding care to obtain agreement.

Correct answer: Do not coerce. **Explanation:** Coercion undermines voluntary consent and may be abusive. **LO: 1 and 4.**

11. **Scenario:** A client's sister requests daily medical updates on a personal WhatsApp number. The client's authorisation is unclear. What should the caregiver do?
A) Start immediately. B) Check consent and approved channels with the supervisor. C) Send only photographs. D) Ask a neighbour.
Correct answer: B. LO: 6.
12. **MCQ:** Minimum-necessary disclosure means:
A) all information to all relatives; B) only relevant information to authorised people for a legitimate purpose; C) no information under any circumstance; D) posting without a name.
Correct answer: B. LO: 6.
13. **Scenario:** A caregiver photographs a wound on a personal phone for "record keeping." What should happen?
A) Send it to family. B) Follow the approved documentation process and report the personal-device use. C) Post it anonymously. D) Keep it as evidence.
Correct answer: B. LO: 6.
14. **Scenario:** A screenshot containing a client's appointment is sent to the wrong group. What should the caregiver do?
A) Delete it and say nothing. B) Report promptly, preserve evidence as instructed and follow privacy-breach procedure. C) Forward a correction with more details. D) Blame the recipient.
Correct answer: B. LO: 6.
15. **True/False:** Removing a client's name from a photograph always makes the client anonymous.
Correct answer: False. Other details can identify the person. **LO: 6.**
16. **What should the caregiver NOT do?** Argue publicly about a complaint using client details.
Correct answer: Use the authorised complaint and incident process. **LO: 6 and 8.**
17. **Scenario:** A client becomes fearful whenever a particular relative enters and has a new unexplained bruise. What is the best response?
A) Accuse the relative immediately. B) Ignore it as a family matter. C) Seek safe communication, protect immediate safety, report and document observations. D) Investigate neighbours.
Correct answer: C. LO: 3–4.
18. **MCQ:** A safeguarding concern is:

A) proof of guilt; B) information that requires safe attention and appropriate reporting; C) a rumour; D) a private investigation.

Correct answer: B. LO: 3.

19. **Scenario:** A client says a relative takes money and asks for secrecy. What should the caregiver do?

A) Promise secrecy. B) Take the bank card. C) Listen, explain limits of confidentiality, assess safety, report and document. D) Confront the relative.

Correct answer: C. LO: 3–4.

20. **Scenario:** A client discloses sexual touching by a caregiver who is due to return. What is the priority?

A) Interview the alleged caregiver. B) Protect immediate safety, report urgently and document exact words. C) Wait for proof. D) Ask the client to post online.

Correct answer: B. LO: 4.

21. **True/False:** The caregiver should investigate to decide whether the client is telling the truth.

Correct answer: False. LO: 3–4.

22. **Scenario:** Several care tasks are repeatedly missed because of staffing problems. What should the caregiver recognise?

A) It cannot be neglect because no one intended harm. B) A possible service-level safeguarding concern requiring protection and escalation. C) A reason to falsify records. D) A family issue.

Correct answer: B. LO: 3 and 8.

23. **Scenario:** A colleague gives one resident private gifts and requests secrecy. What should the caregiver do?

A) Ignore it. B) Join in. C) Recognise possible grooming/boundary risk, record facts and report. D) Publicly accuse the colleague.

Correct answer: C. LO: 3 and 7.

24. **What should the caregiver NOT do after a disclosure?** Promise absolute secrecy, confront the alleged person, interrogate the client or share the story widely.

Correct answer: None of these actions is appropriate. **LO: 3–4.**

25. **Scenario:** A client offers the caregiver an expensive phone and says, “Do not tell the office.” What should the caregiver do?

A) Accept secretly. B) Follow the gift policy, decline or disclose and report the offer. C) Ask for cash. D) Promise to include the client in a will.

Correct answer: B. LO: 7.

26. **Scenario:** A client asks the caregiver to use their bank card and PIN. What should the caregiver do?

A) Memorise the PIN. B) Follow only the authorised money-handling process and seek guidance. C) Use the card for personal shopping too. D) Give the card to a relative.

Correct answer: B. LO: 7.

27. **True/False:** A caregiver may borrow money from a client if repayment is promised.

Correct answer: False. LO: 7.

28. **Scenario:** A client sends the caregiver a friend request and asks for private evening chats. What is the professional response?

A) Accept to maintain trust. B) Decline the personal connection and use approved communication. C) Use a second secret account. D) Share personal problems.

Correct answer: B. LO: 7.

29. **Scenario:** A caregiver could receive a referral payment from a relative's care agency. What should happen?

A) Keep quiet. B) Disclose the conflict and avoid personal recommendation. C) Pressure the client. D) Take payment after referral.

Correct answer: B. LO: 7.

30. **Integrated scenario:** A supervisor asks the caregiver to conceal a client fall. What should the caregiver do?

A) Obey. B) Protect the client, refuse concealment, document and use the alternative escalation route. C) Delete the record. D) Post about it.

Correct answer: B. LO: 8–9.

31. **Scenario:** A serious concern is reported on a form, but no one confirms receipt. What is the best next action?

A) Assume it was read. B) Follow up through the authorised route and escalate if needed. C) Tell unrelated colleagues. D) Wait indefinitely.

Correct answer: B. LO: 9.

32. **MCQ:** Which statement is objective?

A) "The family is evil." B) "Client stated, 'My nephew takes my money' ; two meals were missed; supervisor informed at 15:10." C) "Definitely financial abuse." D) "Everyone knows."

Correct answer: B. LO: 9.

33. **Scenario:** A client's relative asks for a private photograph after the client discloses financial exploitation. What should the caregiver do?
A) Send it. B) Refuse unauthorised sharing, protect the client and report the concern. C) Send it if the relative is older. D) Use a disappearing message.
Correct answer: B. LO: 3, 6 and 9.
34. **What should the caregiver NOT do?** Make a formal legal capacity determination, diagnose abuse, investigate witnesses or promise an outcome.
Correct answer: These actions exceed caregiver scope. **LO:** 3–5 and 9.
35. **Integrated scenario:** The client refuses care, offers money, reports a colleague's inappropriate behaviour and asks that no one be told. List the correct sequence.
Correct answer: Recognise and pause; assess immediate safety and the client's rights; do not accept money; listen without investigating; explain limits of confidentiality; respond within role; report/escalate through the authorised safeguarding and supervisory pathways; document facts objectively; maintain confidentiality and follow up. **LO:** 1–9.

17. Answer key and explanations

Q	Answer	Core reasoning	LO
1	B	Honest reporting and safe follow-up protect the client.	1, 8
2	C	Personal belief cannot override respectful professional duty.	2
3	B	Privacy and explanation make dignity observable.	1
4	B	Falsification creates safety and accountability risk.	8
5	False	Professional warmth does not require private friendship.	1, 7
6	B	Refusal may signal pain or risk; do not	4, 5

		coerce.	
7	B	Consent is voluntary, task-specific and withdrawable.	4
8	C	Avoid lying and refer authorised communication.	5
9	B	Communication difficulty does not remove autonomy.	5
10	Do not coerce	Threats and shame undermine consent.	1, 4
11	B	Authorisation and approved channels must be checked.	6
12	B	Share relevant information only with authorised recipients.	6
13	B	Personal-device photography creates privacy and security risk.	6
14	B	Prompt breach reporting enables protective action.	6
15	False	Context may still identify a person.	6
16	Use complaint route	Public disclosure compounds the privacy breach.	6, 8
17	C	Notice, protect, report and document without accusation.	3–4
18	B	Concerns require safe attention, not proof or investigation.	3

19	C	Financial risk requires listening, safety, reporting and records.	3–4
20	B	Immediate safety and urgent escalation take priority.	4
21	False	Investigation belongs to authorised professionals.	3–4
22	B	System failure can create neglect and must be escalated.	3, 8
23	C	Secrecy and special gifts can indicate grooming or boundary risk.	3, 7
24	None of the listed actions	They may increase risk or compromise the response.	3–4
25	B	Follow policy; do not accept secret personal benefit.	7
26	B	Money handling requires transparent authorised process.	7
27	False	Borrowing exploits or compromises the position of trust.	7
28	B	Personal social relationships create boundary risk.	7
29	B	Disclosure and neutral process manage the conflict.	7
30	B	Protect, refuse falsification and use alternative escalation.	8–9
31	B	Urgent concerns require confirmed	9

		receipt and follow-up.	
32	B	It separates source, observation, time and action from judgement.	9
33	B	Safeguarding and privacy duties override informal requests.	3, 6, 9
34	Do not do these	They exceed caregiver scope.	3–5, 9
35	SAFE-CARE sequence	Structured reasoning integrates rights, safety, scope and accountability.	1–9

18. Recommended behavioural/practical verification

Because this CPD concerns conduct and judgement, NIC or an employer should offer a separate verification activity for roles where observed behaviour matters. Online completion demonstrates knowledge and scenario reasoning; it does not prove that the learner will always behave ethically under pressure.

Recommended verification options

Method	What it tests	Suggested evidence
Scenario-based oral assessment	Recognition, reasoning, scope and escalation	Assessor rubric and recorded outcome
Role-play	Response to disclosure, refusal, gift or family pressure	Observation checklist
Communication exercise	Calm listening, no promises of secrecy, appropriate language	Assessor notes
Documentation exercise	Objective record and handover	Marked sample record

Digital-privacy exercise	Correct response to image, WhatsApp or wrong-recipient incident	Decision record
Supervisor observation	Conduct in real work, boundaries and reliability	Signed observation form

High-level verification checklist

The learner should be able to:

1. identify the ethical, safeguarding, privacy or boundary issue;
2. state the immediate safety risk;
3. preserve dignity and use respectful language;
4. listen without leading or interrogating;
5. avoid promises of secrecy and unsupported accusations;
6. distinguish caregiver action from matters requiring escalation;
7. refuse gifts, money, unauthorised requests or falsification appropriately;
8. identify the correct reporting route;
9. produce an objective, timely and confidential record;
10. explain how confidentiality will be maintained;
11. identify follow-up actions and possible retaliation or further risk;
12. reflect on how personal values or emotions could influence practice.

A pass should require no critical safety failure, such as accepting a bribe, promising absolute secrecy, confronting an alleged abuser in a way that increases risk, falsifying a record, sharing an unauthorised photograph or making a formal legal/clinical determination outside role.

19. Completion requirements

NIC should issue the credential when the learner has completed all 18 lessons, attempted all module assessments, achieved **80% or higher on the 35-question final assessment**, and acknowledged the scope and confidentiality statement. A failed final assessment may be retaken after review; a practical policy is up to three attempts before supervisor review or re-enrolment.

A separate behavioural verification should be required when an employer needs evidence of observed conduct, disclosure response, safeguarding communication, documentation or digital handling. This may be completed by NIC or an employer using the checklist above. It should be recorded separately from the online certificate.

NIC should retain the learner's name and identity, enrolment and completion dates, curriculum version, lesson and module completion, final score, attempts, certificate identifier, acknowledgement of scope, and practical-verification result where applicable. Records should be protected under NIC's information-governance policy and current applicable requirements. The credential demonstrates completion of structured professional development and scenario-based assessment; it does not prove permanent ethical behaviour, legal competence, clinical competence or specialist safeguarding status.

20. Certificate wording

Certificate title

NIC CPD Micro-Credential: Professional Ethics, Safeguarding & Boundaries in Care

Recommended competency statement

This credential recognises successful completion of NIC continuing professional development in professional ethics, safeguarding and boundaries in care. The learner demonstrated knowledge-based and scenario-based understanding of dignity, respect, autonomy, consent, confidentiality, privacy, safeguarding recognition, professional boundaries, financial safeguards, ethical decision-making, reporting, documentation and escalation within caregiver scope of practice.

The certificate should state the 4–5 CPD learning hours, completion date, assessment pass mark, certificate identifier and curriculum version. It should include the following limitation: **“This CPD micro-credential is not a regulated licence, clinical qualification or specialist safeguarding qualification, and does not replace employer policy, supervision or applicable law.”**

21. Digital badge specification

Field	Recommended value
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Badge name	NIC CPD Professional Ethics, Safeguarding & Boundaries in Care
Description	Recognises completion of NIC CPD focused on ethical judgement, dignity, consent, confidentiality, safeguarding recognition, professional boundaries, financial safeguards, reporting and objective documentation.
Issuer	National Institute of Caregivers Nigeria
CPD hours	4–5 learning hours
Level	Professional CPD; intermediate workplace application
Competencies	Ethics; client rights; consent; autonomy; privacy; confidentiality; safeguarding; boundary management; gifts/money; conflicts of interest; reporting and accountability
Evidence requirements	18 lessons, module assessments, 35-question final assessment at 80% and scope acknowledgement; practical verification where separately required
Verification information	NIC verification URL or QR code, certificate ID, completion date and curriculum version
Limitation	Not a regulated licence or specialist safeguarding qualification
Review/renewal	Recommend NIC review at least every 24 months or sooner when relevant law, policy or reporting pathways change

22. NIC portal course description

Professional Ethics, Safeguarding & Boundaries in Care is a 4–5 hour NIC CPD micro-credential for caregivers and direct-care professionals who already understand the fundamentals of caregiving. It develops the judgement required to protect client dignity, rights, privacy and safety in everyday practice.

Through realistic home-care, residential-care, community and healthcare-support scenarios, learners practise responding to consent and refusal, family pressure, possible abuse or neglect, financial exploitation, gifts, private messaging, photography, social media, conflicts of interest, missed care and inaccurate documentation. The course teaches learners to recognise concerns, pause, consider rights and safety, respond within scope, report or escalate, document objectively and maintain confidentiality.

The credential demonstrates successful completion of structured online professional development and scenario-based assessment. It does not make the learner a nurse, clinician, investigator, legal authority or specialist safeguarding professional. Employer supervision, current policies, applicable law and separate behavioural verification remain important.

23. References

- [1] World Health Organization, WHO launches first ever Patient Safety Rights Charter, 18 April 2024.
- [2] World Health Organization, Abuse of older people, 15 June 2024.
- [3] Social Care Institute for Excellence, What is safeguarding?; international practice reference, accessed August 2026.
- [4] Nigeria Data Protection Commission, Resources: Nigeria Data Protection Act, 2023 and related official privacy resources, accessed August 2026.
- [5] World Health Organization, Patient Safety Rights Charter, 2024.
- [6] World Health Organization, Patient safety, accessed August 2026.
- [7] World Health Organization, Tackling abuse of older people: five priorities for the United Nations Decade of Healthy Ageing (2021–2030), 2022.

Source-use statement: WHO and SCIE materials in this curriculum are used as international, evidence-informed practice references. NDPC material is used to identify Nigeria’ s official data-protection framework and resources. NIC and employers must verify current Nigerian laws, state or facility procedures, emergency contacts, safeguarding routes and privacy requirements before publication. No Nigerian reporting obligation, legal conclusion or accreditation equivalence is invented or implied by this curriculum.

24. Quality assurance review

Quality question	Finding	Implementation action
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CPD value	The course assumes prior caregiver knowledge and focuses on judgement, boundaries, safeguarding and accountability.	Keep the prerequisite visible and cross-link foundational NIC courses instead of duplicating them.
Non-duplication	Basic caregiving is not retaught; foundational concepts are brief and immediately applied.	Remove any portal page that becomes a generic introduction to caregiving.
Ethical depth	Learners repeatedly weigh dignity, safety, rights, consent, role and honesty in scenarios.	Preserve scenario feedback, not just answer letters.
Safeguarding	Abuse, neglect, exploitation, discriminatory harm, inappropriate restraint, abandonment, self-neglect and grooming indicators are addressed.	Add current local reporting contacts and safeguarding leads before launch.
Response	The SAFE-CARE framework teaches recognition, safety, listening, reporting, documentation and confidentiality.	Require staff to practise the framework in a role-play or oral verification where appropriate.
Boundaries	Gifts, money, loans, inheritance, social media, private messages, transport, personal disclosure, emotional dependence, romance, favouritism and conflicts are included.	Link the course to each employer' s gift, money and social-media policy.
Digital professionalism	Photography, video, WhatsApp, screenshots, geolocation, wrong-recipient messages and public complaints are covered.	Specify approved devices, systems and breach contacts in portal implementation.
Cultural relevance	Family involvement, multigenerational households, faith, community relationships and hospitality are treated respectfully without stereotyping.	Use local examples reviewed by Nigerian caregiver educators and safeguarding professionals.
Scope	Caregivers are told not to diagnose, investigate,	Make the scope statement mandatory at enrolment and

	determine legal capacity, adjudicate guilt or manage assets for personal benefit.	completion.
Assessment quality	27 of 35 final questions are scenario/application based, exceeding the required minimum of 23.	Retain plausible but clearly inappropriate distractors and explain why they are unsafe.
Professional credibility	Completion requirements, practical verification, certificate limitations and evidence retention are specified.	Obtain formal NIC subject-matter and governance approval before publication.
Source accuracy	References are traceable and distinguish international guidance from Nigerian context and employer policy.	Review references and local pathways at least every 24 months or after material changes.

Final implementation checklist

Before launch, NIC should confirm that the portal contains the six-module hierarchy, 18 lessons, module assessments, final assessment, certificate and badge workflow. Each employer or service using the course should insert current contact details for the supervisor, safeguarding lead, emergency route, privacy/data-protection contact, complaints channel and alternative escalation route where the ordinary supervisor is involved.

NIC should also review the course for accessibility, plain-language clarity, translation needs, trauma-informed presentation, secure assessment handling, data retention, practical-verification arrangements and compatibility with current Nigerian requirements. The completed credential should communicate the intended professional standard: **the learner understands the trust placed in them, can recognise threats to dignity, rights or safety, can maintain appropriate boundaries and confidentiality, and knows when and how to report or escalate a concern.**